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## **Title 42 —Public Health**

### **Chapter IV —Centers for Medicare & Medicaid Services, Department of Health and Human Services**

#### **Subchapter B —Medicare Program**

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## PART 418—HOSPICE CARE

**Authority:** 42 U.S.C. 1302 and 1395hh.

**Source:** 48 FR 56026, Dec. 16, 1983, unless otherwise noted.

### Subpart A—General Provision and Definitions

#### § 418.1 Statutory basis.

This part implements section 1861(dd) of the Social Security Act (the Act). Section 1861(dd) of the Act specifies services covered as hospice care and the conditions that a hospice program must meet in order to participate in the Medicare program. Section 1861(dd) also specifies limitations on coverage of, and payment for, inpatient hospice care. The following sections of the Act are also pertinent:

- (a) Sections 1812(a) (4) and (d) of the Act specify eligibility requirements for the individual and the benefit periods.
- (b) Section 1813(a)(4) of the Act specifies coinsurance amounts.
- (c) Sections 1814(a)(7) and 1814(i) of the Act contain conditions and limitations on coverage of, and payment for, hospice care.
- (d) Sections 1862(a) (1), (6) and (9) of the Act establish limits on hospice coverage.

[48 FR 56026, Dec. 16, 1983, as amended at 57 FR 36017, Aug. 12, 1992; 74 FR 39413, Aug. 6, 2009]

#### § 418.2 Scope of part.

Subpart A of this part sets forth the statutory basis and scope and defines terms used in this part. Subpart B specifies the eligibility and election requirements and the benefit periods. Subparts C and D specify the conditions of participation for hospices. Subpart E is reserved for future use. Subparts F and G specify coverage and payment policy. Subpart H specifies coinsurance amounts applicable to hospice care.

[74 FR 39413, Aug. 6, 2009]

#### § 418.3 Definitions.

For purposes of this part—

*Attending physician* means a—

- (1)
  - (i) Doctor of medicine or osteopathy legally authorized to practice medicine and surgery by the State in which he or she performs that function or action; or
  - (ii) Nurse practitioner who meets the training, education, and experience requirements as described in § 410.75(b) of this chapter; or
  - (iii) Physician assistant who meets the requirements of § 410.74(c) of this chapter.

- (2) Is identified by the individual, at the time he or she elects to receive hospice care, as having the most significant role in the determination and delivery of the individual's medical care.

**Bereavement counseling** means emotional, psychosocial, and spiritual support and services provided before and after the death of the patient to assist with issues related to grief, loss, and adjustment.

**BFCC-QIO** means Beneficiary and Family Centered Care Quality Improvement Organization.

**Cap period** means the twelve-month period ending September 30 used in the application of the cap on overall hospice reimbursement specified in § 418.309.

**Clinical note** means a notation of a contact with the patient and/or the family that is written and dated by any person providing services and that describes signs and symptoms, treatments and medications administered, including the patient's reaction and/or response, and any changes in physical, emotional, psychosocial or spiritual condition during a given period of time.

**Comprehensive assessment** means a thorough evaluation of the patient's physical, psychosocial, emotional and spiritual status related to the terminal illness and related conditions. This includes a thorough evaluation of the caregiver's and family's willingness and capability to care for the patient.

**Dietary counseling** means education and interventions provided to the patient and family regarding appropriate nutritional intake as the patient's condition progresses. Dietary counseling is provided by qualified individuals, which may include a registered nurse, dietitian or nutritionist, when identified in the patient's plan of care.

**Employee** means a person who:

- (1) Works for the hospice and for whom the hospice is required to issue a W-2 form on his or her behalf;
- (2) If the hospice is a subdivision of an agency or organization, an employee of the agency or organization who is assigned to the hospice; or
- (3) Is a volunteer under the jurisdiction of the hospice.

**Hospice** means a public agency or private organization or subdivision of either of these that is primarily engaged in providing hospice care as defined in this section.

**Hospice care** means a comprehensive set of services described in 1861(dd)(1) of the Act, identified and coordinated by an interdisciplinary group to provide for the physical, psychosocial, spiritual, and emotional needs of a terminally ill patient and/or family members, as delineated in a specific patient plan of care.

**Initial assessment** means an evaluation of the patient's physical, psychosocial and emotional status related to the terminal illness and related conditions to determine the patient's immediate care and support needs.

**Licensed professional** means a person licensed to provide patient care services by the State in which services are delivered.

**Multiple location** means a Medicare-approved location from which the hospice provides the same full range of hospice care and services that is required of the hospice issued the certification number. A multiple location must meet all of the conditions of participation applicable to hospices.

**Palliative care** means patient and family-centered care that optimizes quality of life by anticipating, preventing, and treating suffering. Palliative care throughout the continuum of illness involves addressing physical, intellectual, emotional, social, and spiritual needs and to facilitate patient autonomy, access to information, and choice.

**Physician** means an individual who meets the qualifications and conditions as defined in section 1861(r) of the Act and implemented at § 410.20 of this chapter.

**Physician designee** means a doctor of medicine or osteopathy designated by the hospice who assumes the same responsibilities and obligations as the medical director when the medical director is not available.

**Pseudo-patient** means a person trained to participate in a role-play situation, or a computer-based mannequin device. A pseudo-patient must be capable of responding to and interacting with the hospice aide trainee, and must demonstrate the general characteristics of the primary patient population served by the hospice in key areas such as age, frailty, functional status, cognitive status and care goals.

**Representative** means an individual who has the authority under State law (whether by statute or pursuant to an appointment by the courts of the State) to authorize or terminate medical care or to elect or revoke the election of hospice care on behalf of a terminally ill patient who is mentally or physically incapacitated. This may include a legal guardian.

**Restraint** means—

- (1) Any manual method, physical or mechanical device, material, or equipment that immobilizes or reduces the ability of a patient to move his or her arms, legs, body, or head freely, not including devices, such as orthopedically prescribed devices, surgical dressings or bandages, protective helmets, or other methods that involve the physical holding of a patient for the purpose of conducting routine physical examinations or tests, or to protect the patient from falling out of bed, or to permit the patient to participate in activities without the risk of physical harm (this does not include a physical escort); or
- (2) A drug or medication when it is used as a restriction to manage the patient's behavior or restrict the patient's freedom of movement and is not a standard treatment or dosage for the patient's condition.

**Seclusion** means the involuntary confinement of a patient alone in a room or an area from which the patient is physically prevented from leaving.

**Simulation** means a training and assessment technique that mimics the reality of the homecare environment, including environmental distractions and constraints that evoke or replicate substantial aspects of the real world in a fully interactive fashion, in order to teach and assess proficiency in performing skills, and to promote decision making and critical thinking.

**Terminally ill** means that the individual has a medical prognosis that his or her life expectancy is 6 months or less if the illness runs its normal course.

[48 FR 56026, Dec. 16, 1983, as amended at 52 FR 4499, Feb. 12, 1987; 55 FR 50834, Dec. 11, 1990; 70 FR 45144, Aug. 4, 2005; 72 FR 50227, Aug. 31, 2007; 73 FR 32204, June 5, 2008; 79 FR 50509, Aug. 22, 2014; 83 FR 38654, Aug. 6, 2018; 84 FR 38543, Aug. 6, 2019; 86 FR 42605, Aug. 4, 2021]

## Subpart B—Eligibility, Election and Duration of Benefits

## § 418.20 Eligibility requirements.

In order to be eligible to elect hospice care under Medicare, an individual must be—

- (a) Entitled to Part A of Medicare; and
- (b) Certified as being terminally ill in accordance with § 418.22.

## § 418.21 Duration of hospice care coverage—Election periods.

- (a) Subject to the conditions set forth in this part, an individual may elect to receive hospice care during one or more of the following election periods:
  - (1) An initial 90-day period;
  - (2) A subsequent 90-day period; or
  - (3) An unlimited number of subsequent 60-day periods.
- (b) The periods of care are available in the order listed and may be elected separately at different times.

[55 FR 50834, Dec. 11, 1990, as amended at 57 FR 36017, Aug. 12, 1992; 70 FR 70546, Nov. 22, 2005]

## § 418.22 Certification of terminal illness.

- (a) *Timing of certification —*
  - (1) **General rule.** The hospice must obtain written certification of terminal illness for each of the periods listed in § 418.21, even if a single election continues in effect for an unlimited number of periods, as provided in § 418.24(c).
  - (2) **Basic requirement.** Except as provided in paragraph (a)(3) of this section, the hospice must obtain the written certification before it submits a claim for payment.
  - (3) **Exceptions.**
    - (i) If the hospice cannot obtain the written certification within 2 calendar days, after a period begins, it must obtain an oral certification within 2 calendar days and the written certification before it submits a claim for payment.
    - (ii) Certifications may be completed no more than 15 calendar days prior to the effective date of election.
    - (iii) Recertifications may be completed no more than 15 calendar days prior to the start of the subsequent benefit period.
  - (4) **Face-to-face encounter.**
    - (i) As of January 1, 2011, a hospice physician or hospice nurse practitioner must have a face-to-face encounter with each hospice patient whose total stay across all hospices is anticipated to reach the 3rd benefit period. The face-to-face encounter must occur prior to, but no more than 30 calendar days prior to, the 3rd benefit period recertification, and every benefit period recertification thereafter, to gather clinical findings to determine continued eligibility for hospice care.

- (ii) During a Public Health Emergency, as defined in § 400.200 of this chapter, or through September 30, 2025, whichever is later, if the face-to-face encounter conducted by a hospice physician or hospice nurse practitioner is for the sole purpose of hospice recertification, such encounter may occur via a telecommunications technology and is considered an administrative expense. *Telecommunications technology* means the use of interactive multimedia communications equipment that includes, at a minimum, the use of audio and video equipment permitting two-way, real-time interactive communication between the patient and the distant site hospice physician or hospice nurse practitioner.
- (b) **Content of certification.** Certification will be based on the physician's or medical director's clinical judgment regarding the normal course of the individual's illness. The certification must conform to the following requirements:
- (1) The certification must specify that the individual's prognosis is for a life expectancy of 6 months or less if the terminal illness runs its normal course.
  - (2) Clinical information and other documentation that support the medical prognosis must accompany the certification and must be filed in the medical record with the written certification as set forth in paragraph (d)(2) of this section. Initially, the clinical information may be provided verbally, and must be documented in the medical record and included as part of the hospice's eligibility assessment.
  - (3) The physician must include a brief narrative explanation of the clinical findings that supports a life expectancy of 6 months or less as part of the certification and recertification forms, or as an addendum to the certification and recertification forms.
    - (i) If the narrative is part of the certification or recertification form, then the narrative must be located immediately prior to the physician's signature.
    - (ii) If the narrative exists as an addendum to the certification or recertification form, in addition to the physician's signature on the certification or recertification form, the physician must also sign immediately following the narrative in the addendum.
    - (iii) The narrative shall include a statement directly above the physician signature attesting that by signing, the physician confirms that he/she composed the narrative based on his/her review of the patient's medical record or, if applicable, his/her examination of the patient.
    - (iv) The narrative must reflect the patient's individual clinical circumstances and cannot contain check boxes or standard language used for all patients.
    - (v) The narrative associated with the 3rd benefit period recertification and every subsequent recertification must include an explanation of why the clinical findings of the face-to-face encounter support a life expectancy of 6 months or less.
  - (4) The physician or nurse practitioner who performs the face-to-face encounter with the patient described in paragraph (a)(4) of this section must attest in writing that he or she had a face-to-face encounter with the patient, including the date of that visit. The attestation must include the physician's or nurse practitioner's signature and the date it was signed. The attestation could be a separate and distinct section of, or an addendum to, the recertification or a clinical note that indicates the face-to-face encounter occurred, and includes the clinical findings of the face-to-face encounter, the date of the visit, the signature of the physician or nurse practitioner who conducted the face-to-face encounter, and the date of the signature. If the attestation of the nurse practitioner

or a non-certifying hospice physician is a separate and distinct section of, or an addendum to, the recertification, the attestation shall state that the clinical findings of that visit were provided to the certifying physician for use in determining continued eligibility for hospice care.

- (5) All certifications and recertifications must be signed and dated by the physician(s), and must include the benefit period dates to which the certification or recertification applies.

(c) **Sources of certification.**

- (1) For the initial 90-day period, the hospice must obtain written certification statements (and oral certification statements if required under paragraph (a)(3) of this section) from—
  - (i) The medical director of the hospice, the physician designee (as defined in § 418.3), or the physician member of the hospice interdisciplinary group; and
  - (ii) The individual's attending physician, if the individual has an attending physician. The attending physician must meet the definition of physician specified in § 410.20 of this subchapter.
- (2) For subsequent periods, the only requirement is certification by one of the physicians listed in paragraph (c)(1)(i) of this section.

(d) **Maintenance of records.** Hospice staff must—

- (1) Make an appropriate entry in the patient's medical record as soon as they receive an oral certification; and
- (2) File written certifications in the medical record.

[55 FR 50834, Dec. 11, 1990, as amended at 57 FR 36017, Aug. 12, 1992; 70 FR 45144, Aug. 4, 2005; 70 FR 70547, Nov. 22, 2005; 74 FR 39413, Aug. 6, 2009; 75 FR 70463, Nov. 17, 2010; 76 FR 47331, Aug. 4, 2011; 85 FR 19289, Apr. 6, 2020; 88 FR 51199, Aug. 2, 2023; 89 FR 64272, Aug. 6, 2024; 90 FR 37430, Aug. 5, 2025]

## § 418.24 Election of hospice care.

- (a) **Election statement.** An individual who meets the eligibility requirement of § 418.20 may file an election statement with a particular hospice. If the individual is physically or mentally incapacitated, his or her representative (as defined in § 418.3) may file the election statement.
- (b) **Content of election statement.** The election statement must include the following:
  - (1) Identification of the particular hospice and of the attending physician that will provide care to the individual. The individual or representative must acknowledge that the identified attending physician was his or her choice.
  - (2) The individual's or representative's acknowledgement that he or she has been given a full understanding of the palliative rather than curative nature of hospice care, as it relates to the individual's terminal illness and related conditions.
  - (3) Acknowledgement that the individual has been provided information on the hospice's coverage responsibility and that certain Medicare services, as set forth in paragraph (g) of this section, are waived by the election. For Hospice elections beginning on or after October 1, 2020, this would include providing the individual with information indicating that services unrelated to the terminal illness and related conditions are exceptional and unusual and hospice should be providing virtually all care needed by the individual who has elected hospice.



- (4) The effective date of the election, which may be the first day of hospice care or a later date, but may be no earlier than the date of the election statement.
  - (5) For Hospice elections beginning on or after October 1, 2020, the Hospice must provide information on individual cost-sharing for hospice services.
  - (6) For Hospice elections beginning on or after October 1, 2020, the Hospice must provide notification of the individual's (or representative's) right to receive an election statement addendum, as set forth in paragraph (c) of this section, if there are conditions, items, services, and drugs the hospice has determined to be unrelated to the individual's terminal illness and related conditions and would not be covered by the hospice.
  - (7) For Hospice elections beginning on or after October 1, 2020, the Hospice must provide information on the Beneficiary and Family Centered Care Quality Improvement Organization (BFCC-QIO), including the right to immediate advocacy and BFCC-QIO contact information.
  - (8) The signature of the individual or representative.
- (c) **Content of hospice election statement addendum.** For hospice elections beginning on or after October 1, 2020, in the event that the hospice determines there are conditions, items, services, or drugs that are unrelated to the individual's terminal illness and related conditions, the individual (or representative), non-hospice providers furnishing such items, services, or drugs, or Medicare contractors may request a written list as an addendum to the election statement. The election statement addendum must include the following:
- (1) The addendum must be titled "Patient Notification of Hospice Non-Covered Items, Services, and Drugs."
  - (2) Name of the hospice.
  - (3) Individual's name and hospice medical record identifier.
  - (4) Identification of the individual's terminal illness and related conditions.
  - (5) A list of the individual's conditions present on hospice admission (or upon plan of care update) and the associated items, services, and drugs not covered by the hospice because they have been determined by the hospice to be unrelated to the terminal illness and related conditions.
  - (6) A written clinical explanation, in language the individual (or representative) can understand, as to why the identified conditions, items, services, and drugs are considered unrelated to the individual's terminal illness and related conditions and not needed for pain or symptom management. This clinical explanation must be accompanied by a general statement that the decision as to whether or not conditions, items, services, and drugs are related is made for each patient and that the individual should share this clinical explanation with other health care providers from which they seek items, services, or drugs unrelated to their terminal illness and related conditions.
  - (7) References to any relevant clinical practice, policy, or coverage guidelines.
  - (8) Information on the following:
    - (i) **Purpose of Addendum.** The purpose of the addendum is to notify the individual (or representative), in writing, of those conditions, items, services, and drugs the hospice will not be covering because the hospice has determined they are unrelated to the individual's terminal illness and related conditions.

(ii) **Right to Immediate Advocacy.** The addendum must include language that immediate advocacy is available through the Medicare Beneficiary and Family Centered Care-Quality Improvement Organization (BFCC-QIO) if the individual (or representative) disagrees with the hospice's determination.

(9) Name and signature of the individual (or representative) and date signed, along with a statement that signing this addendum (or its updates) is only acknowledgement of receipt of the addendum (or its updates) and not the individual's (or representative's) agreement with the hospice's determinations. If the beneficiary (or representative) refuses to sign the addendum, the hospice must document on the addendum the reason the addendum was not signed and the addendum would become part of the patient's medical record. If a non-hospice provider or Medicare contractor requests the addendum, the non-hospice provider or Medicare contractor are not required to sign the addendum.

(10) Date the hospice furnished the addendum.

(d) **Timeframes for the hospice election statement addendum.**

(1) If the addendum is requested within the first 5 days of a hospice election (that is, in the first 5 days of the hospice election date), the hospice must provide this information, in writing, to the individual (or representative), non-hospice provider, or Medicare contractor within 5 days from the date of the request.

(2) If the addendum is requested during the course of hospice care (that is, after the first 5 days of the hospice election date), the hospice must provide this information, in writing, within 3 days of the request to the requesting individual (or representative), non-hospice provider, or Medicare contractor.

(3) If there are any changes to the plan of care during the course of hospice care, the hospice must update the addendum and provide these updates, in writing, to the individual (or representative) in order to communicate these changes to the individual (or representative).

(4) If the individual dies, revokes, or is discharged within the required timeframe for furnishing the addendum (as outlined in paragraphs (d)(1) and (2) of this section, and before the hospice has furnished the addendum, the addendum would not be required to be furnished to the individual (or representative). The hospice must note the reason the addendum was not furnished to the patient and the addendum would become part of the patient's medical record if the hospice has completed it at the time of discharge, revocation, or death.

(5) If the beneficiary dies, revokes, or is discharged prior to signing the addendum (as outlined in paragraphs (d)(1) and (2) of this section), the addendum would not be required to be signed in order for the hospice to receive payment. The hospice must note (on the addendum itself) the reason the addendum was not signed and the addendum would become part of the patient's medical record.

(e) **Notice of election.** The hospice chosen by the eligible individual (or his or her representative) must file the Notice of Election (NOE) with its Medicare contractor within 5 calendar days after the effective date of the election statement.

(1) **Consequences of failure to submit a timely notice of election.** When a hospice does not file the required Notice of Election for its Medicare patients within 5 calendar days after the effective date of election, Medicare will not cover and pay for days of hospice care from the effective date of election to the date of filing of the notice of election. These days are a provider liability, and the provider may not bill the beneficiary for them.

- (2) **Exception to the consequences for filing the NOE late.** CMS may waive the consequences of failure to submit a timely-filed NOE specified in paragraph (e)(1) of this section. CMS will determine if a circumstance encountered by a hospice is exceptional and qualifies for waiver of the consequence specified in paragraph (e)(1) of this section. A hospice must fully document and furnish any requested documentation to CMS for a determination of exception. An exceptional circumstance may be due to, but is not limited to, the following:
  - (i) Fires, floods, earthquakes, or similar unusual events that inflict extensive damage to the hospice's ability to operate.
  - (ii) A CMS or Medicare contractor systems issue that is beyond the control of the hospice.
  - (iii) A newly Medicare-certified hospice that is notified of that certification after the Medicare certification date, or which is awaiting its user ID from its Medicare contractor.
  - (iv) Other situations determined by CMS to be beyond the control of the hospice.
- (f) **Duration of election.** An election to receive hospice care will be considered to continue through the initial election period and through the subsequent election periods without a break in care as long as the individual—
  - (1) Remains in the care of a hospice;
  - (2) Does not revoke the election; and
  - (3) Is not discharged from the hospice under the provisions of § 418.26.
- (g) **Waiver of other benefits.** For the duration of an election of hospice care, an individual waives all rights to Medicare payments for the following services:
  - (1) Hospice care provided by a hospice other than the hospice designated by the individual (unless provided under arrangements made by the designated hospice).
  - (2) Any Medicare services that are related to the treatment of the terminal condition for which hospice care was elected or a related condition or that are equivalent to hospice care except for services—
    - (i) Provided by the designated hospice;
    - (ii) Provided by another hospice under arrangements made by the designated hospice; and
    - (iii) Provided by the individual's attending physician if that physician is not an employee of the designated hospice or receiving compensation from the hospice for those services.
- (h) **Re-election of hospice benefits.** If an election has been revoked in accordance with § 418.28, the individual (or his or her representative if the individual is mentally or physically incapacitated) may at any time file an election, in accordance with this section, for any other election period that is still available to the individual.
- (i) **Changing the attending physician.** To change the designated attending physician, the individual (or representative) must file a signed statement with the hospice that states that he or she is changing his or her attending physician.
  - (1) The statement must identify the new attending physician, and include the date the change is to be effective and the date signed by the individual (or representative).

- (2) The individual (or representative) must acknowledge that the change in the attending physician is due to his or her choice.
- (3) The effective date of the change in attending physician cannot be before the date the statement is signed.

[55 FR 50834, Dec. 11, 1990, as amended at 70 FR 70547, Nov. 22, 2005; 79 FR 50509, Aug. 22, 2014; 84 FR 38544, Aug. 6, 2019; 86 FR 42605, Aug. 4, 2021; 89 FR 64272, Aug. 6, 2024]

### § 418.25 Admission to hospice care.

- (a) The hospice admits a patient only on the recommendation of the medical director (or the physician designee, as defined in § 418.3) or the physician member of the hospice interdisciplinary group, in consultation with, or with input from, the patient's attending physician (if any).
- (b) In reaching a decision to certify that the patient is terminally ill, the hospice medical director (or the physician designee, as defined in § 418.3) or the physician member of the hospice interdisciplinary group, must consider at least the following information:
  - (1) Diagnosis of the terminal condition of the patient.
  - (2) Other health conditions, whether related or unrelated to the terminal condition.
  - (3) Current clinically relevant information supporting all diagnoses.

[70 FR 70547, Nov. 22, 2005, as amended at 89 FR 64272, Aug. 6, 2024; 90 FR 37431, Aug. 5, 2025]

### § 418.26 Discharge from hospice care.

- (a) **Reasons for discharge.** A hospice may discharge a patient if—
  - (1) The patient moves out of the hospice's service area or transfers to another hospice;
  - (2) The hospice determines that the patient is no longer terminally ill; or
  - (3) The hospice determines, under a policy set by the hospice for the purpose of addressing discharge for cause that meets the requirements of paragraphs (a)(3)(i) through (a)(3)(iv) of this section, that the patient's (or other persons in the patient's home) behavior is disruptive, abusive, or uncooperative to the extent that delivery of care to the patient or the ability of the hospice to operate effectively is seriously impaired. The hospice must do the following before it seeks to discharge a patient for cause:
    - (i) Advise the patient that a discharge for cause is being considered;
    - (ii) Make a serious effort to resolve the problem(s) presented by the patient's behavior or situation;
    - (iii) Ascertain that the patient's proposed discharge is not due to the patient's use of necessary hospice services; and
    - (iv) Document the problem(s) and efforts made to resolve the problem(s) and enter this documentation into its medical records.

- (b) **Discharge order.** Prior to discharging a patient for any reason listed in paragraph (a) of this section, the hospice must obtain a written physician's discharge order from the hospice medical director. If a patient has an attending physician involved in his or her care, this physician should be consulted before discharge and his or her review and decision included in the discharge note.
- (c) **Effect of discharge.** An individual, upon discharge from the hospice during a particular election period for reasons other than immediate transfer to another hospice—
  - (1) Is no longer covered under Medicare for hospice care;
  - (2) Resumes Medicare coverage of the benefits waived under § 418.24(e); and
  - (3) May at any time elect to receive hospice care if he or she is again eligible to receive the benefit.
- (d) **Discharge planning.**
  - (1) The hospice must have in place a discharge planning process that takes into account the prospect that a patient's condition might stabilize or otherwise change such that the patient cannot continue to be certified as terminally ill.
  - (2) The discharge planning process must include planning for any necessary family counseling, patient education, or other services before the patient is discharged because he or she is no longer terminally ill.
- (e) **Filing a notice of termination of election.** When the hospice election is ended due to discharge, the hospice must file a notice of termination/revocation of election with its Medicare contractor within 5 calendar days after the effective date of the discharge, unless it has already filed a final claim for that beneficiary.

[70 FR 70547, Nov. 22, 2005, as amended at 79 FR 50509, Aug. 22, 2014; 84 FR 38544, Aug. 6, 2019]

## § 418.28 Revoking the election of hospice care.

- (a) An individual or representative may revoke the individual's election of hospice care at any time during an election period.
- (b) To revoke the election of hospice care, the individual or representative must file a statement with the hospice that includes the following information:
  - (1) A signed statement that the individual or representative revokes the individual's election for Medicare coverage of hospice care for the remainder of that election period.
  - (2) The date that the revocation is to be effective. (An individual or representative may not designate an effective date earlier than the date that the revocation is made).
- (c) An individual, upon revocation of the election of Medicare coverage of hospice care for a particular election period—
  - (1) Is no longer covered under Medicare for hospice care;
  - (2) Resumes Medicare coverage of the benefits waived under § 418.24(f)(2); and
  - (3) May at any time elect to receive hospice coverage for any other hospice election periods that he or she is eligible to receive.

- (d) When the hospice election is ended due to revocation, the hospice must file a notice of termination/ revocation of election with its Medicare contractor within 5 calendar days after the effective date of the revocation, unless it has already filed a final claim for that beneficiary.

[48 FR 56026, Dec. 16, 1983, as amended at 79 FR 50509, Aug. 22, 2014; 84 FR 38544, Aug. 6, 2019]

### § 418.30 Change of the designated hospice.

- (a) An individual or representative may change, once in each election period, the designation of the particular hospice from which hospice care will be received.
- (b) The change of the designated hospice is not a revocation of the election for the period in which it is made.
- (c) To change the designation of hospice programs, the individual or representative must file, with the hospice from which care has been received and with the newly designated hospice, a statement that includes the following information:
  - (1) The name of the hospice from which the individual has received care and the name of the hospice from which he or she plans to receive care.
  - (2) The date the change is to be effective.

### Subpart C—Conditions of Participation: Patient Care

**Source:** 73 FR 32204, June 5, 2008, unless otherwise noted.

### § 418.52 Condition of participation: Patient's rights.

The patient has the right to be informed of his or her rights, and the hospice must protect and promote the exercise of these rights.

- (a) **Standard: Notice of rights and responsibilities.**
  - (1) During the initial assessment visit in advance of furnishing care the hospice must provide the patient or representative with verbal (meaning spoken) and written notice of the patient's rights and responsibilities in a language and manner that the patient understands.
  - (2) The hospice must comply with the requirements of subpart I of part 489 of this chapter regarding advance directives. The hospice must inform and distribute written information to the patient concerning its policies on advance directives, including a description of applicable State law.
  - (3) The hospice must obtain the patient's or representative's signature confirming that he or she has received a copy of the notice of rights and responsibilities.
- (b) **Standard: Exercise of rights and respect for property and person.**
  - (1) The patient has the right:
    - (i) To exercise his or her rights as a patient of the hospice;
    - (ii) To have his or her property and person treated with respect;

- (iii) To voice grievances regarding treatment or care that is (or fails to be) furnished and the lack of respect for property by anyone who is furnishing services on behalf of the hospice; and
- (iv) To not be subjected to discrimination or reprisal for exercising his or her rights.
- (2) If a patient has been adjudged incompetent under state law by a court of proper jurisdiction, the rights of the patient are exercised by the person appointed pursuant to state law to act on the patient's behalf.
- (3) If a state court has not adjudged a patient incompetent, any legal representative designated by the patient in accordance with state law may exercise the patient's rights to the extent allowed by state law.
- (4) The hospice must:
  - (i) Ensure that all alleged violations involving mistreatment, neglect, or verbal, mental, sexual, and physical abuse, including injuries of unknown source, and misappropriation of patient property by anyone furnishing services on behalf of the hospice, are reported immediately by hospice employees and contracted staff to the hospice administrator;
  - (ii) Immediately investigate all alleged violations involving anyone furnishing services on behalf of the hospice and immediately take action to prevent further potential violations while the alleged violation is being verified. Investigations and/or documentation of all alleged violations must be conducted in accordance with established procedures;
  - (iii) Take appropriate corrective action in accordance with state law if the alleged violation is verified by the hospice administration or an outside body having jurisdiction, such as the State survey agency or local law enforcement agency; and
  - (iv) Ensure that verified violations are reported to State and local bodies having jurisdiction (including to the State survey and certification agency) within 5 working days of becoming aware of the violation.

(c) **Standard: Rights of the patient.** The patient has a right to the following:

- (1) Receive effective pain management and symptom control from the hospice for conditions related to the terminal illness;
- (2) Be involved in developing his or her hospice plan of care;
- (3) Refuse care or treatment;
- (4) Choose his or her attending physician;
- (5) Have a confidential clinical record. Access to or release of patient information and clinical records is permitted in accordance with 45 CFR parts 160 and 164.
- (6) Be free from mistreatment, neglect, or verbal, mental, sexual, and physical abuse, including injuries of unknown source, and misappropriation of patient property;
- (7) Receive information about the services covered under the hospice benefit;
- (8) Receive information about the scope of services that the hospice will provide and specific limitations on those services.

### § 418.54 Condition of participation: Initial and comprehensive assessment of the patient.

The hospice must conduct and document in writing a patient-specific comprehensive assessment that identifies the patient's need for hospice care and services, and the patient's need for physical, psychosocial, emotional, and spiritual care. This assessment includes all areas of hospice care related to the palliation and management of the terminal illness and related conditions.

- (a) **Standard: Initial assessment.** The hospice registered nurse must complete an initial assessment within 48 hours after the election of hospice care in accordance with § 418.24 is complete (unless the physician, patient, or representative requests that the initial assessment be completed in less than 48 hours.)
- (b) **Standard: Timeframe for completion of the comprehensive assessment.** The hospice interdisciplinary group, in consultation with the individual's attending physician (if any), must complete the comprehensive assessment no later than 5 calendar days after the election of hospice care in accordance with § 418.24.
- (c) **Standard: Content of the comprehensive assessment.** The comprehensive assessment must identify the physical, psychosocial, emotional, and spiritual needs related to the terminal illness that must be addressed in order to promote the hospice patient's well-being, comfort, and dignity throughout the dying process. The comprehensive assessment must take into consideration the following factors:
  - (1) The nature and condition causing admission (including the presence or lack of objective data and subjective complaints).
  - (2) Complications and risk factors that affect care planning.
  - (3) Functional status, including the patient's ability to understand and participate in his or her own care.
  - (4) Imminence of death.
  - (5) Severity of symptoms.
  - (6) **Drug profile.** A review of all of the patient's prescription and over-the-counter drugs, herbal remedies and other alternative treatments that could affect drug therapy. This includes, but is not limited to, identification of the following:
    - (i) Effectiveness of drug therapy.
    - (ii) Drug side effects.
    - (iii) Actual or potential drug interactions.
    - (iv) Duplicate drug therapy.
    - (v) Drug therapy currently associated with laboratory monitoring.
  - (7) **Bereavement.** An initial bereavement assessment of the needs of the patient's family and other individuals focusing on the social, spiritual, and cultural factors that may impact their ability to cope with the patient's death. Information gathered from the initial bereavement assessment must be incorporated into the plan of care and considered in the bereavement plan of care.
  - (8) The need for referrals and further evaluation by appropriate health professionals.
- (d) **Standard: Update of the comprehensive assessment.** The update of the comprehensive assessment must be accomplished by the hospice interdisciplinary group (in collaboration with the individual's attending physician, if any) and must consider changes that have taken place since the initial assessment. It must



include information on the patient's progress toward desired outcomes, as well as a reassessment of the patient's response to care. The assessment update must be accomplished as frequently as the condition of the patient requires, but no less frequently than every 15 days.

(e) **Standard: Patient outcome measures.**

- (1) The comprehensive assessment must include data elements that allow for measurement of outcomes. The hospice must measure and document data in the same way for all patients. The data elements must take into consideration aspects of care related to hospice and palliation.
- (2) The data elements must be an integral part of the comprehensive assessment and must be documented in a systematic and retrievable way for each patient. The data elements for each patient must be used in individual patient care planning and in the coordination of services, and must be used in the aggregate for the hospice's quality assessment and performance improvement program.

**§ 418.56 Condition of participation: Interdisciplinary group, care planning, and coordination of services.**

The hospice must designate an interdisciplinary group or groups as specified in paragraph (a) of this section which, in consultation with the patient's attending physician, must prepare a written plan of care for each patient. The plan of care must specify the hospice care and services necessary to meet the patient and family-specific needs identified in the comprehensive assessment as such needs relate to the terminal illness and related conditions.

(a) **Standard: Approach to service delivery.**

- (1) The hospice must designate an interdisciplinary group or groups composed of individuals who work together to meet the physical, medical, psychosocial, emotional, and spiritual needs of the hospice patients and families facing terminal illness and bereavement. Interdisciplinary group members must provide the care and services offered by the hospice, and the group, in its entirety, must supervise the care and services. The hospice must designate a registered nurse that is a member of the interdisciplinary group to provide coordination of care and to ensure continuous assessment of each patient's and family's needs and implementation of the interdisciplinary plan of care. The interdisciplinary group must include, but is not limited to, individuals who are qualified and competent to practice in the following professional roles:
  - (i) A doctor of medicine or osteopathy (who is an employee or under contract with the hospice).
  - (ii) A registered nurse.
  - (iii) A social worker, marriage and family therapist, or a mental health counselor.
  - (iv) A pastoral or other counselor.
- (2) If the hospice has more than one interdisciplinary group, it must identify a specifically designated interdisciplinary group to establish policies governing the day-to-day provision of hospice care and services.

(b) **Standard: Plan of care.** All hospice care and services furnished to patients and their families must follow an individualized written plan of care established by the hospice interdisciplinary group in collaboration with the attending physician (if any), the patient or representative, and the primary caregiver in

accordance with the patient's needs if any of them so desire. The hospice must ensure that each patient and the primary care giver(s) receive education and training provided by the hospice as appropriate to their responsibilities for the care and services identified in the plan of care.

- (c) **Standard: Content of the plan of care.** The hospice must develop an individualized written plan of care for each patient. The plan of care must reflect patient and family goals and interventions based on the problems identified in the initial, comprehensive, and updated comprehensive assessments. The plan of care must include all services necessary for the palliation and management of the terminal illness and related conditions, including the following:
  - (1) Interventions to manage pain and symptoms.
  - (2) A detailed statement of the scope and frequency of services necessary to meet the specific patient and family needs.
  - (3) Measurable outcomes anticipated from implementing and coordinating the plan of care.
  - (4) Drugs and treatment necessary to meet the needs of the patient.
  - (5) Medical supplies and appliances necessary to meet the needs of the patient.
  - (6) The interdisciplinary group's documentation of the patient's or representative's level of understanding, involvement, and agreement with the plan of care, in accordance with the hospice's own policies, in the clinical record.
- (d) **Standard: Review of the plan of care.** The hospice interdisciplinary group (in collaboration with the individual's attending physician, if any) must review, revise and document the individualized plan as frequently as the patient's condition requires, but no less frequently than every 15 calendar days. A revised plan of care must include information from the patient's updated comprehensive assessment and must note the patient's progress toward outcomes and goals specified in the plan of care.
- (e) **Standard: Coordination of services.** The hospice must develop and maintain a system of communication and integration, in accordance with the hospice's own policies and procedures, to—
  - (1) Ensure that the interdisciplinary group maintains responsibility for directing, coordinating, and supervising the care and services provided.
  - (2) Ensure that the care and services are provided in accordance with the plan of care.
  - (3) Ensure that the care and services provided are based on all assessments of the patient and family needs.
  - (4) Provide for and ensure the ongoing sharing of information between all disciplines providing care and services in all settings, whether the care and services are provided directly or under arrangement.
  - (5) Provide for an ongoing sharing of information with other non-hospice healthcare providers furnishing services unrelated to the terminal illness and related conditions.

[73 FR 32204, June 5, 2008, as amended at 88 FR 79539, Nov. 16, 2023]

#### **§ 418.58 Condition of participation: Quality assessment and performance improvement.**

The hospice must develop, implement, and maintain an effective, ongoing, hospice-wide data-driven quality assessment and performance improvement program. The hospice's governing body must ensure that the program: Reflects the complexity of its organization and services; involves all hospice services (including those services

furnished under contract or arrangement); focuses on indicators related to improved palliative outcomes; and takes actions to demonstrate improvement in hospice performance. The hospice must maintain documentary evidence of its quality assessment and performance improvement program and be able to demonstrate its operation to CMS.

(a) **Standard: Program scope.**

- (1) The program must at least be capable of showing measurable improvement in indicators related to improved palliative outcomes and hospice services.
- (2) The hospice must measure, analyze, and track quality indicators, including adverse patient events, and other aspects of performance that enable the hospice to assess processes of care, hospice services, and operations.

(b) **Standard: Program data.**

- (1) The program must use quality indicator data, including patient care, and other relevant data, in the design of its program.
- (2) The hospice must use the data collected to do the following:
  - (i) Monitor the effectiveness and safety of services and quality of care.
  - (ii) Identify opportunities and priorities for improvement.
- (3) The frequency and detail of the data collection must be approved by the hospice's governing body.

(c) **Standard: Program activities.**

- (1) The hospice's performance improvement activities must:
  - (i) Focus on high risk, high volume, or problem-prone areas.
  - (ii) Consider incidence, prevalence, and severity of problems in those areas.
  - (iii) Affect palliative outcomes, patient safety, and quality of care.
- (2) Performance improvement activities must track adverse patient events, analyze their causes, and implement preventive actions and mechanisms that include feedback and learning throughout the hospice.
- (3) The hospice must take actions aimed at performance improvement and, after implementing those actions, the hospice must measure its success and track performance to ensure that improvements are sustained.

(d) **Standard: Performance improvement projects.** Beginning February 2, 2009 hospices must develop, implement, and evaluate performance improvement projects.

- (1) The number and scope of distinct performance improvement projects conducted annually, based on the needs of the hospice's population and internal organizational needs, must reflect the scope, complexity, and past performance of the hospice's services and operations.
- (2) The hospice must document what performance improvement projects are being conducted, the reasons for conducting these projects, and the measurable progress achieved on these projects.

(e) **Standard: Executive responsibilities.** The hospice's governing body is responsible for ensuring the following:

- (1) That an ongoing program for quality improvement and patient safety is defined, implemented, and maintained, and is evaluated annually.
- (2) That the hospice-wide quality assessment and performance improvement efforts address priorities for improved quality of care and patient safety, and that all improvement actions are evaluated for effectiveness.
- (3) That one or more individual(s) who are responsible for operating the quality assessment and performance improvement program are designated.

#### § 418.60 Condition of participation: Infection control.

The hospice must maintain and document an effective infection control program that protects patients, families, visitors, and hospice personnel by preventing and controlling infections and communicable diseases.

- (a) **Standard: Prevention.** The hospice must follow accepted standards of practice to prevent the transmission of infections and communicable diseases, including the use of standard precautions.
- (b) **Standard: Control.** The hospice must maintain a coordinated agency-wide program for the surveillance, identification, prevention, control, and investigation of infectious and communicable diseases that—
  - (1) Is an integral part of the hospice's quality assessment and performance improvement program; and
  - (2) Includes the following:
    - (i) A method of identifying infectious and communicable disease problems; and
    - (ii) A plan for implementing the appropriate actions that are expected to result in improvement and disease prevention.
- (c) **Standard: Education.** The hospice must provide infection control education to employees, contracted providers, patients, and family members and other caregivers.

[73 FR 32204, June 5, 2008, as amended at 86 FR 61616, Nov. 5, 2021; 88 FR 36510, June 5, 2023]

#### § 418.62 Condition of participation: Licensed professional services.

- (a) Licensed professional services provided directly or under arrangement must be authorized, delivered, and supervised only by health care professionals who meet the appropriate qualifications specified under § 418.114 and who practice under the hospice's policies and procedures.
- (b) Licensed professionals must actively participate in the coordination of all aspects of the patient's hospice care, in accordance with current professional standards and practice, including participating in ongoing interdisciplinary comprehensive assessments, developing and evaluating the plan of care, and contributing to patient and family counseling and education; and
- (c) Licensed professionals must participate in the hospice's quality assessment and performance improvement program and hospice sponsored in-service training.

#### CORE SERVICES

### § 418.64 Condition of participation: Core services.

A hospice must routinely provide substantially all core services directly by hospice employees. These services must be provided in a manner consistent with acceptable standards of practice. These services include nursing services, medical social services, and counseling. The hospice may contract for physician services as specified in paragraph (a) of this section. A hospice may use contracted staff, if necessary, to supplement hospice employees in order to meet the needs of patients under extraordinary or other non-routine circumstances. A hospice may also enter into a written arrangement with another Medicare certified hospice program for the provision of core services to supplement hospice employee/staff to meet the needs of patients. Circumstances under which a hospice may enter into a written arrangement for the provision of core services include: Unanticipated periods of high patient loads, staffing shortages due to illness or other short-term temporary situations that interrupt patient care; and temporary travel of a patient outside of the hospice's service area.

- (a) **Standard: Physician services.** The hospice medical director, physician employees, and contracted physician(s) of the hospice, in conjunction with the patient's attending physician, are responsible for the palliation and management of the terminal illness and conditions related to the terminal illness.
  - (1) All physician employees and those under contract, must function under the supervision of the hospice medical director.
  - (2) All physician employees and those under contract shall meet this requirement by either providing the services directly or through coordinating patient care with the attending physician.
  - (3) If the attending physician is unavailable, the medical director, contracted physician, and/or hospice physician employee is responsible for meeting the medical needs of the patient.
- (b) **Standard: Nursing services.**
  - (1) The hospice must provide nursing care and services by or under the supervision of a registered nurse. Nursing services must ensure that the nursing needs of the patient are met as identified in the patient's initial assessment, comprehensive assessment, and updated assessments.
  - (2) If State law permits registered nurses to see, treat, and write orders for patients, then registered nurses may provide services to beneficiaries receiving hospice care.
  - (3) Highly specialized nursing services that are provided so infrequently that the provision of such services by direct hospice employees would be impracticable and prohibitively expensive, may be provided under contract.
- (c) **Standard: Medical social services.** Medical social services must be provided by a qualified social worker, under the direction of a physician. Social work services must be based on the patient's psychosocial assessment and the patient's and family's needs and acceptance of these services.
- (d) **Standard: Counseling services.** Counseling services must be available to the patient and family to assist the patient and family in minimizing the stress and problems that arise from the terminal illness, related conditions, and the dying process. Counseling services must include, but are not limited to, the following:
  - (1) **Bereavement counseling.** The hospice must:
    - (i) Have an organized program for the provision of bereavement services furnished under the supervision of a qualified professional with experience or education in grief or loss counseling.

- (ii) Make bereavement services available to the family and other individuals in the bereavement plan of care up to 1 year following the death of the patient. Bereavement counseling also extends to residents of a SNF/NF or ICF/IID when appropriate and identified in the bereavement plan of care.
  - (iii) Ensure that bereavement services reflect the needs of the bereaved.
  - (iv) Develop a bereavement plan of care that notes the kind of bereavement services to be offered and the frequency of service delivery. A special coverage provision for bereavement counseling is specified in § 418.204(c).
- (2) **Dietary counseling.** Dietary counseling, when identified in the plan of care, must be performed by a qualified individual, which include dietitians as well as nurses and other individuals who are able to address and assure that the dietary needs of the patient are met.
- (3) **Spiritual counseling.** The hospice must:
  - (i) Provide an assessment of the patient's and family's spiritual needs.
  - (ii) Provide spiritual counseling to meet these needs in accordance with the patient's and family's acceptance of this service, and in a manner consistent with patient and family beliefs and desires.
  - (iii) Make all reasonable efforts to facilitate visits by local clergy, pastoral counselors, or other individuals who can support the patient's spiritual needs to the best of its ability.
  - (iv) Advise the patient and family of this service.

**§ 418.66 Condition of participation: Nursing services—Waiver of requirement that substantially all nursing services be routinely provided directly by a hospice.**

- (a) CMS may waive the requirement in § 418.64(b) that a hospice provide nursing services directly, if the hospice is located in a non-urbanized area. The location of a hospice that operates in several areas is considered to be the location of its central office. The hospice must provide evidence to CMS that it has made a good faith effort to hire a sufficient number of nurses to provide services. CMS may waive the requirement that nursing services be furnished by employees based on the following criteria:
  - (1) The location of the hospice's central office is in a non-urbanized area as determined by the Bureau of the Census.
  - (2) There is evidence that a hospice was operational on or before January 1, 1983 including the following:
    - (i) Proof that the organization was established to provide hospice services on or before January 1, 1983.
    - (ii) Evidence that hospice-type services were furnished to patients on or before January 1, 1983.
    - (iii) Evidence that hospice care was a discrete activity rather than an aspect of another type of provider's patient care program on or before January 1, 1983.
  - (3) By virtue of the following evidence that a hospice made a good faith effort to hire nurses:
    - (i) Copies of advertisements in local newspapers that demonstrate recruitment efforts.
    - (ii) Job descriptions for nurse employees.

- (iii) Evidence that salary and benefits are competitive for the area.
- (iv) Evidence of any other recruiting activities (for example, recruiting efforts at health fairs and contacts with nurses at other providers in the area).
- (b) Any waiver request is deemed to be granted unless it is denied within 60 days after it is received.
- (c) Waivers will remain effective for 1 year at a time from the date of the request.
- (d) If a hospice wishes to receive a 1-year extension, it must submit a request to CMS before the expiration of the waiver period, and certify that the conditions under which it originally requested the initial waiver have not changed since the initial waiver was granted.

#### NON-CORE SERVICES

### **§ 418.70 Condition of participation: Furnishing of non-core services.**

A hospice must ensure that the services described in § 418.72 through § 418.78 are provided directly by the hospice or under arrangements made by the hospice as specified in § 418.100. These services must be provided in a manner consistent with current standards of practice.

### **§ 418.72 Condition of participation: Physical therapy, occupational therapy, and speech-language pathology.**

Physical therapy services, occupational therapy services, and speech-language pathology services must be available, and when provided, offered in a manner consistent with accepted standards of practice.

### **§ 418.74 Waiver of requirement—Physical therapy, occupational therapy, speech-language pathology, and dietary counseling.**

- (a) A hospice located in a non-urbanized area may submit a written request for a waiver of the requirement for providing physical therapy, occupational therapy, speech-language pathology, and dietary counseling services. The hospice may seek a waiver of the requirement that it make physical therapy, occupational therapy, speech-language pathology, and dietary counseling services (as needed) available on a 24-hour basis. The hospice may also seek a waiver of the requirement that it provide dietary counseling directly. The hospice must provide evidence that it has made a good faith effort to meet the requirements for these services before it seeks a waiver. CMS may approve a waiver application on the basis of the following criteria:
  - (1) The hospice is located in a non-urbanized area as determined by the Bureau of the Census.
  - (2) The hospice provides evidence that it had made a good faith effort to make available physical therapy, occupational therapy, speech-language pathology, and dietary counseling services on a 24-hour basis and/or to hire a dietary counselor to furnish services directly. This evidence must include the following:
    - (i) Copies of advertisements in local newspapers that demonstrate recruitment efforts.
    - (ii) Physical therapy, occupational therapy, speech-language pathology, and dietary counselor job descriptions.
    - (iii) Evidence that salary and benefits are competitive for the area.

- (iv) Evidence of any other recruiting activities (for example, recruiting efforts at health fairs and contact discussions with physical therapy, occupational therapy, speech-language pathology, and dietary counseling service providers in the area).
- (b) Any waiver request is deemed to be granted unless it is denied within 60 days after it is received.
- (c) An initial waiver will remain effective for 1 year at a time from the date of the request.
- (d) If a hospice wishes to receive a 1-year extension, it must submit a request to CMS before the expiration of the waiver period and certify that conditions under which it originally requested the waiver have not changed since the initial waiver was granted.

### **§ 418.76 Condition of participation: Hospice aide and homemaker services.**

All hospice aide services must be provided by individuals who meet the personnel requirements specified in paragraph (a) of this section. Homemaker services must be provided by individuals who meet the personnel requirements specified in paragraph (j) of this section.

- (a) **Standard: Hospice aide qualifications.**
  - (1) A qualified hospice aide is a person who has successfully completed one of the following:
    - (i) A training program and competency evaluation as specified in paragraphs (b) and (c) of this section respectively.
    - (ii) A competency evaluation program that meets the requirements of paragraph (c) of this section.
    - (iii) A nurse aide training and competency evaluation program approved by the State as meeting the requirements of § 483.151 through § 483.154 of this chapter, and is currently listed in good standing on the State nurse aide registry.
    - (iv) A State licensure program.
  - (2) A hospice aide is not considered to have completed a program, as specified in paragraph (a)(1) of this section, if, since the individual's most recent completion of the program(s), there has been a continuous period of 24 consecutive months during which none of the services furnished by the individual as described in § 409.40 of this chapter were for compensation. If there has been a 24-month lapse in furnishing services, the individual must complete another program, as specified in paragraph (a)(1) of this section, before providing services.
- (b) **Standard: Content and duration of hospice aide classroom and supervised practical training.**
  - (1) Hospice aide training must include classroom and supervised practical training in a practicum laboratory or other setting in which the trainee demonstrates knowledge while performing tasks on an individual under the direct supervision of a registered nurse, or a licensed practical nurse, who is under the supervision of a registered nurse. Classroom and supervised practical training combined must total at least 75 hours.
  - (2) A minimum of 16 hours of classroom training must precede a minimum of 16 hours of supervised practical training as part of the 75 hours.
  - (3) A hospice aide training program must address each of the following subject areas:
    - (i) Communication skills, including the ability to read, write, and verbally report clinical information to patients, care givers, and other hospice staff.



- (ii) Observation, reporting, and documentation of patient status and the care or service furnished.
- (iii) Reading and recording temperature, pulse, and respiration.
- (iv) Basic infection control procedures.
- (v) Basic elements of body functioning and changes in body function that must be reported to an aide's supervisor.
- (vi) Maintenance of a clean, safe, and healthy environment.
- (vii) Recognizing emergencies and the knowledge of emergency procedures and their application.
- (viii) The physical, emotional, and developmental needs of and ways to work with the populations served by the hospice, including the need for respect for the patient, his or her privacy, and his or her property.
- (ix) Appropriate and safe techniques in performing personal hygiene and grooming tasks, including items on the following basic checklist:
  - (A) Bed bath.
  - (B) Sponge, tub, and shower bath.
  - (C) Hair shampoo (sink, tub, and bed).
  - (D) Nail and skin care.
  - (E) Oral hygiene.
  - (F) Toileting and elimination.
- (x) Safe transfer techniques and ambulation.
- (xi) Normal range of motion and positioning.
- (xii) Adequate nutrition and fluid intake.
- (xiii) Any other task that the hospice may choose to have an aide perform. The hospice is responsible for training hospice aides, as needed, for skills not covered in the basic checklist, as described in paragraph (b)(3)(ix) of this section.

(4) The hospice must maintain documentation that demonstrates that the requirements of this standard are met.

(c) **Standard: Competency evaluation.** An individual may furnish hospice aide services on behalf of a hospice only after that individual has successfully completed a competency evaluation program as described in this section.

(1) The competency evaluation must address each of the subjects listed in paragraph (b)(3) of this section. Subject areas specified under paragraphs (b)(3)(i), (iii), (ix), (x), and (xi) of this section must be evaluated by observing an aide's performance of the task with a patient or pseudo-patient. The remaining subject areas may be evaluated through written examination, oral examination, or after observation of a hospice aide with a patient or a pseudo-patient during a simulation.

(2) A hospice aide competency evaluation program may be offered by any organization, except as described in paragraph (f) of this section.

- (3) The competency evaluation must be performed by a registered nurse in consultation with other skilled professionals, as appropriate.
- (4) A hospice aide is not considered competent in any task for which he or she is evaluated as unsatisfactory. An aide must not perform that task without direct supervision by a registered nurse until after he or she has received training in the task for which he or she was evaluated as "unsatisfactory," and successfully completes a subsequent evaluation. A hospice aide is not considered to have successfully completed a competency evaluation if the aide has an "unsatisfactory" rating in more than one of the required areas.
- (5) The hospice must maintain documentation that demonstrates the requirements of this standard are being met.
- (d) **Standard: In-service training.** A hospice aide must receive at least 12 hours of in-service training during each 12-month period. In-service training may occur while an aide is furnishing care to a patient.
  - (1) In-service training may be offered by any organization, and must be supervised by a registered nurse.
  - (2) The hospice must maintain documentation that demonstrates the requirements of this standard are met.
- (e) **Standard: Qualifications for instructors conducting classroom and supervised practical training.** Classroom and supervised practical training must be performed by a registered nurse who possesses a minimum of 2 years nursing experience, at least 1 year of which must be in home care, or by other individuals under the general supervision of a registered nurse.
- (f) **Standard: Eligible competency evaluation organizations.** A hospice aide competency evaluation program as specified in paragraph (c) of this section may be offered by any organization except by a home health agency that, within the previous 2 years:
  - (1) Had been out of compliance with the requirements of § 484.80 of this chapter.
  - (2) Permitted an individual that does not meet the definition of a "qualified home health aide" as specified in § 484.80(a) of this chapter to furnish home health aide services (with the exception of licensed health professionals and volunteers).
  - (3) Had been subjected to an extended (or partial extended) survey as a result of having been found to have furnished substandard care (or for other reasons at the discretion of CMS or the State).
  - (4) Had been assessed a civil monetary penalty of \$5,000 or more as an intermediate sanction.
  - (5) Had been found by CMS to have compliance deficiencies that endangered the health and safety of the home health agency's patients and had temporary management appointed to oversee the management of the home health agency.
  - (6) Had all or part of its Medicare payments suspended.
  - (7) Had been found by CMS or the State under any Federal or State law to have:
    - (i) Had its participation in the Medicare program terminated.
    - (ii) Been assessed a penalty of \$5,000 or more for deficiencies in Federal or State standards for home health agencies.
    - (iii) Been subjected to a suspension of Medicare payments to which it otherwise would have been entitled.

- (iv) Operated under temporary management that was appointed by a governmental authority to oversee the operation of the home health agency and to ensure the health and safety of the home health agency's patients.
- (v) Been closed by CMS or the State, or had its patients transferred by the State.

(g) **Standard: Hospice aide assignments and duties.**

- (1) Hospice aides are assigned to a specific patient by a registered nurse that is a member of the interdisciplinary group. Written patient care instructions for a hospice aide must be prepared by a registered nurse who is responsible for the supervision of a hospice aide as specified under paragraph (h) of this section.
- (2) A hospice aide provides services that are:
  - (i) Ordered by the interdisciplinary group.
  - (ii) Included in the plan of care.
  - (iii) Permitted to be performed under State law by such hospice aide.
  - (iv) Consistent with the hospice aide training.
- (3) The duties of a hospice aide include the following:
  - (i) The provision of hands-on personal care.
  - (ii) The performance of simple procedures as an extension of therapy or nursing services.
  - (iii) Assistance in ambulation or exercises.
  - (iv) Assistance in administering medications that are ordinarily self-administered.
- (4) Hospice aides must report changes in the patient's medical, nursing, rehabilitative, and social needs to a registered nurse, as the changes relate to the plan of care and quality assessment and improvement activities. Hospice aides must also complete appropriate records in compliance with the hospice's policies and procedures.

(h) **Standard: Supervision of hospice aides.**

- (1) A registered nurse must make an on-site visit to the patient's home:
  - (i) No less frequently than every 14 days to assess the quality of care and services provided by the hospice aide and to ensure that services ordered by the hospice interdisciplinary group meet the patient's needs. The hospice aide does not have to be present during this visit.
  - (ii) If an area of concern is noted by the supervising nurse, then the hospice must make an on-site visit to the location where the patient is receiving care in order to observe and assess the aide while he or she is performing care.
  - (iii) If an area of concern is verified by the hospice during the on-site visit, then the hospice must conduct, and the hospice aide must complete, a competency evaluation of the deficient skill and all related skill(s) in accordance with paragraph (c) of this section.
- (2) A registered nurse must make an annual on-site visit to the location where a patient is receiving care in order to observe and assess each aide while he or she is performing care.

- (3) The supervising nurse must assess an aide's ability to demonstrate initial and continued satisfactory performance in meeting outcome criteria that include, but is not limited to—
  - (i) Following the patient's plan of care for completion of tasks assigned to the hospice aide by the registered nurse.
  - (ii) Creating successful interpersonal relationships with the patient and family.
  - (iii) Demonstrating competency with assigned tasks.
  - (iv) Complying with infection control policies and procedures.
  - (v) Reporting changes in the patient's condition.
- (i) **Standard: Individuals furnishing Medicaid personal care aide-only services under a Medicaid personal care benefit.** An individual may furnish personal care services, as defined in § 440.167 of this chapter, on behalf of a hospice agency.
  - (1) Before the individual may furnish personal care services, the individual must be found competent by the State (if regulated by the State) to furnish those services. The individual only needs to demonstrate competency in the services the individual is required to furnish.
  - (2) Services under the Medicaid personal care benefit may be used to the extent that the hospice would routinely use the services of a hospice patient's family in implementing a patient's plan of care.
  - (3) The hospice must coordinate its hospice aide and homemaker services with the Medicaid personal care benefit to ensure the patient receives the hospice aide and homemaker services he or she needs.
- (j) **Standard: Homemaker qualifications.** A qualified homemaker is—
  - (1) An individual who meets the standards in § 418.202(g) and has successfully completed hospice orientation addressing the needs and concerns of patients and families coping with a terminal illness; or
  - (2) A hospice aide as described in § 418.76.
- (k) **Standard: Homemaker supervision and duties.**
  - (1) Homemaker services must be coordinated and supervised by a member of the interdisciplinary group.
  - (2) Instructions for homemaker duties must be prepared by a member of the interdisciplinary group.
  - (3) Homemakers must report all concerns about the patient or family to the member of the interdisciplinary group who is coordinating homemaker services.

[73 FR 32204, June 5, 2008, as amended at 74 FR 39413, Aug. 6, 2009; 82 FR 4578, Jan. 13, 2017; 84 FR 51815, Sept. 30, 2019; 86 FR 42605, Aug. 4, 2021]

### § 418.78 Conditions of participation—Volunteers.

The hospice must use volunteers to the extent specified in paragraph (e) of this section. These volunteers must be used in defined roles and under the supervision of a designated hospice employee.

- (a) **Standard: Training.** The hospice must maintain, document, and provide volunteer orientation and training that is consistent with hospice industry standards.
- (b) **Standard: Role.** Volunteers must be used in day-to-day administrative and/or direct patient care roles.
- (c) **Standard: Recruiting and retaining.** The hospice must document and demonstrate viable and ongoing efforts to recruit and retain volunteers.
- (d) **Standard: Cost saving.** The hospice must document the cost savings achieved through the use of volunteers. Documentation must include the following:
  - (1) The identification of each position that is occupied by a volunteer.
  - (2) The work time spent by volunteers occupying those positions.
  - (3) Estimates of the dollar costs that the hospice would have incurred if paid employees occupied the positions identified in paragraph (d)(1) of this section for the amount of time specified in paragraph (d)(2) of this section.
- (e) **Standard: Level of activity.** Volunteers must provide day-to-day administrative and/or direct patient care services in an amount that, at a minimum, equals 5 percent of the total patient care hours of all paid hospice employees and contract staff. The hospice must maintain records on the use of volunteers for patient care and administrative services, including the type of services and time worked.

## Subpart D—Conditions of participation: Organizational Environment

**Source:** 73 FR 32204, June 5, 2008, unless otherwise noted.

### § 418.100 Condition of Participation: Organization and administration of services.

The hospice must organize, manage, and administer its resources to provide the hospice care and services to patients, caregivers and families necessary for the palliation and management of the terminal illness and related conditions.

- (a) **Standard: Serving the hospice patient and family.** The hospice must provide hospice care that—
  - (1) Optimizes comfort and dignity; and
  - (2) Is consistent with patient and family needs and goals, with patient needs and goals as priority.
- (b) **Standard: Governing body and administrator.** A governing body (or designated persons so functioning) assumes full legal authority and responsibility for the management of the hospice, the provision of all hospice services, its fiscal operations, and continuous quality assessment and performance improvement. A qualified administrator appointed by and reporting to the governing body is responsible for the day-to-day operation of the hospice. The administrator must be a hospice employee and possess education and experience required by the hospice's governing body.
- (c) **Standard: Services.**
  - (1) A hospice must be primarily engaged in providing the following care and services and must do so in a manner that is consistent with accepted standards of practice:
    - (i) Nursing services.

- (ii) Medical social services.
  - (iii) Physician services.
  - (iv) Counseling services, including spiritual counseling, dietary counseling, and bereavement counseling.
  - (v) Hospice aide, volunteer, and homemaker services.
  - (vi) Physical therapy, occupational therapy, and speech-language pathology services.
  - (vii) Short-term inpatient care.
  - (viii) Medical supplies (including drugs and biologicals) and medical appliances.
- (2) Nursing services, physician services, and drugs and biologicals (as specified in § 418.106) must be made routinely available on a 24-hour basis 7 days a week. Other covered services must be available on a 24-hour basis when reasonable and necessary to meet the needs of the patient and family.
- (d) **Standard: Continuation of care.** A hospice may not discontinue or reduce care provided to a Medicare or Medicaid beneficiary because of the beneficiary's inability to pay for that care.
- (e) **Standard: Professional management responsibility.** A hospice that has a written agreement with another agency, individual, or organization to furnish any services under arrangement must retain administrative and financial management, and oversight of staff and services for all arranged services, to ensure the provision of quality care. Arranged services must be supported by written agreements that require that all services be—
- (1) Authorized by the hospice;
  - (2) Furnished in a safe and effective manner by qualified personnel; and
  - (3) Delivered in accordance with the patient's plan of care.
- (f) **Standard: Hospice multiple locations.** If a hospice operates multiple locations, it must meet the following requirements:
- (1) Medicare approval.
    - (i) All hospice multiple locations must be approved by Medicare before providing hospice care and services to Medicare patients.
    - (ii) The multiple location must be part of the hospice and must share administration, supervision, and services with the hospice issued the certification number.
    - (iii) The lines of authority and professional and administrative control must be clearly delineated in the hospice's organizational structure and in practice, and must be traced to the location which was issued the certification number.
    - (iv) The determination that a multiple location does or does not meet the definition of a multiple location, as set forth in this part, is an initial determination, as set forth in § 498.3.
  - (2) The hospice must continually monitor and manage all services provided at all of its locations to ensure that services are delivered in a safe and effective manner and to ensure that each patient and family receives the necessary care and services outlined in the plan of care, in accordance with the requirements of this subpart and subparts A and C of this section.

(g) **Standard: Training.**

- (1) A hospice must provide orientation about the hospice philosophy to all employees and contracted staff who have patient and family contact.
- (2) A hospice must provide an initial orientation for each employee that addresses the employee's specific job duties.
- (3) A hospice must assess the skills and competence of all individuals furnishing care, including volunteers furnishing services, and, as necessary, provide in-service training and education programs where required. The hospice must have written policies and procedures describing its method(s) of assessment of competency and maintain a written description of the in-service training provided during the previous 12 months.

[73 FR 32204, June 5, 2008, as amended at 74 FR 39413, Aug. 6, 2009]

**§ 418.102 Condition of participation: Medical director.**

The hospice must designate a physician to serve as medical director. The medical director must be a doctor of medicine or osteopathy who is an employee or is under contract with the hospice. When the medical director is not available, a physician designee as defined at § 418.3 assumes the same responsibilities and obligations as the medical director.

(a) **Standard: Medical director contract.**

- (1) A hospice may contract with either of the following—
  - (i) A self-employed physician; or
  - (ii) A physician employed by a professional entity or physicians group. When contracting for medical director services, the contract must specify the physician who assumes the medical director responsibilities and obligations.

(b) **Standard: Initial certification of terminal illness.** The medical director (or physician designee, as defined in § 418.3, if the medical director is unavailable) or physician member of the IDG reviews the clinical information for each hospice patient and provides written certification that it is anticipated that the patient's life expectancy is 6 months or less if the illness runs its normal course. The physician must consider the following when making this determination:

- (1) The primary terminal condition;
- (2) Related diagnosis(es), if any;
- (3) Current subjective and objective medical findings;
- (4) Current medication and treatment orders; and
- (5) Information about the medical management of any of the patient's conditions unrelated to the terminal illness.

(c) **Standard: Recertification of the terminal illness.** Before each recertification period for each patient, as described in § 418.21(a), the medical director (or physician designee, as defined in § 418.3, if the medical director is unavailable) or physician member of the IDG must review the patient's clinical information.

- (d) **Standard: Medical director responsibility.** The medical director or physician designee has responsibility for the medical component of the hospice's patient care program.

[73 FR 32204, June 5, 2008, as amended at 89 FR 64272, Aug. 6, 2024]

#### § 418.104 Condition of participation: Clinical records.

A clinical record containing past and current findings is maintained for each hospice patient. The clinical record must contain correct clinical information that is available to the patient's attending physician and hospice staff. The clinical record may be maintained electronically.

- (a) **Standard: Content.** Each patient's record must include the following:
  - (1) The initial plan of care, updated plans of care, initial assessment, comprehensive assessment, updated comprehensive assessments, and clinical notes.
  - (2) Signed copies of the notice of patient rights in accordance with § 418.52 and election statement in accordance with § 418.24.
  - (3) Responses to medications, symptom management, treatments, and services.
  - (4) Outcome measure data elements, as described in § 418.54(e) of this subpart.
  - (5) Physician certification and recertification of terminal illness as required in §§ 418.22 and 418.25 and described in §§ 418.102(b) and 418.102(c) respectively, if appropriate.
  - (6) Any advance directives as described in § 418.52(a)(2).
  - (7) Physician orders.
- (b) **Standard: Authentication.** All entries must be legible, clear, complete, and appropriately authenticated and dated in accordance with hospice policy and currently accepted standards of practice.
- (c) **Standard: Protection of information.** The clinical record, its contents and the information contained therein must be safeguarded against loss or unauthorized use. The hospice must be in compliance with the Department's rules regarding personal health information as set out at 45 CFR parts 160 and 164.
- (d) **Standard: Retention of records.** Patient clinical records must be retained for 6 years after the death or discharge of the patient, unless State law stipulates a longer period of time. If the hospice discontinues operation, hospice policies must provide for retention and storage of clinical records. The hospice must inform its State agency and its CMS Regional office where such clinical records will be stored and how they may be accessed.
- (e) **Standard: Discharge or transfer of care.**
  - (1) If the care of a patient is transferred to another Medicare/Medicaid-certified facility, the hospice must forward to the receiving facility, a copy of—
    - (i) The hospice discharge summary; and
    - (ii) The patient's clinical record, if requested.
  - (2) If a patient revokes the election of hospice care, or is discharged from hospice in accordance with § 418.26, the hospice must forward to the patient's attending physician, a copy of—
    - (i) The hospice discharge summary; and



(ii) The patient's clinical record, if requested.

(3) The hospice discharge summary as required in paragraph (e)(1) and (e)(2) of this section must include—

(i) A summary of the patient's stay including treatments, symptoms and pain management.

(ii) The patient's current plan of care.

(iii) The patient's latest physician orders. and

(iv) Any other documentation that will assist in post-discharge continuity of care or that is requested by the attending physician or receiving facility.

(f) **Standard: Retrieval of clinical records.** The clinical record, whether hard copy or in electronic form, must be made readily available on request by an appropriate authority.

### **§ 418.106 Condition of participation: Drugs and biologicals, medical supplies, and durable medical equipment.**

Medical supplies and appliances, as described in § 410.36 of this chapter; durable medical equipment, as described in § 410.38 of this chapter; and drugs and biologicals related to the palliation and management of the terminal illness and related conditions, as identified in the hospice plan of care, must be provided by the hospice while the patient is under hospice care.

(a) **Standard: Managing drugs and biologicals.**

(1) A hospice that provides inpatient care directly in its own facility must provide pharmacy services under the direction of a qualified licensed pharmacist who is an employee of or under contract with the hospice. The provided pharmacist services must include evaluation of a patient's response to medication therapy, identification of potential adverse drug reactions, and recommended appropriate corrective action.

(2) [Reserved]

(b) **Standard: Ordering of drugs.**

(1) Drugs may be ordered by any of the following practitioners:

(i) A physician as defined by section 1861(r)(1) of the Act.

(ii) A nurse practitioner in accordance with state scope of practice requirements.

(iii) A physician assistant in accordance with state scope of practice requirements and hospice policy who is:

(A) The patient's attending physician; and

(B) Not an employee of or under arrangement with the hospice.

(2) If the drug order is verbal or given by or through electronic transmission—

(i) It must be given only to a licensed nurse, nurse practitioner (where appropriate), pharmacist, or physician; and

(ii) The individual receiving the order must record and sign it immediately and have the prescribing person sign it in accordance with State and Federal regulations.

(c) **Standard: Dispensing of drugs and biologicals.** The hospice must—

- (1) Obtain drugs and biologicals from community or institutional pharmacists or stock drugs and biologicals itself.
- (2) The hospice that provides inpatient care directly in its own facility must:
  - (i) Have a written policy in place that promotes dispensing accuracy; and
  - (ii) Maintain current and accurate records of the receipt and disposition of all controlled drugs.

(d) **Standard: Administration of drugs and biologicals.**

- (1) The interdisciplinary group, as part of the review of the plan of care, must determine the ability of the patient and/or family to safely self-administer drugs and biologicals to the patient in his or her home.
- (2) Patients receiving care in a hospice that provides inpatient care directly in its own facility may only be administered medications by the following individuals:
  - (i) A licensed nurse, physician, or other health care professional in accordance with their scope of practice and State law;
  - (ii) An employee who has completed a State-approved training program in medication administration; and
  - (iii) The patient, upon approval by the interdisciplinary group.

(e) **Standard: Labeling, disposing, and storing of drugs and biologicals —**

- (1) **Labeling.** Drugs and biologicals must be labeled in accordance with currently accepted professional practice and must include appropriate usage and cautionary instructions, as well as an expiration date (if applicable).
- (2) **Disposing.**
  - (i) Safe use and disposal of controlled drugs in the patient's home. The hospice must have written policies and procedures for the management and disposal of controlled drugs in the patient's home. At the time when controlled drugs are first ordered the hospice must:
    - (A) Provide a copy of the hospice written policies and procedures on the management and disposal of controlled drugs to the patient or patient representative and family;
    - (B) Discuss the hospice policies and procedures for managing the safe use and disposal of controlled drugs with the patient or representative and the family in a language and manner that they understand to ensure that these parties are educated regarding the safe use and disposal of controlled drugs; and
    - (C) Document in the patient's clinical record that the written policies and procedures for managing controlled drugs was provided and discussed.
  - (ii) Disposal of controlled drugs in hospices that provide inpatient care directly. The hospice that provides inpatient care directly in its own facility must dispose of controlled drugs in compliance with the hospice policy and in accordance with State and Federal requirements. The hospice must maintain current and accurate records of the receipt and disposition of all controlled drugs.

- (3) **Storing.** The hospice that provides inpatient care directly in its own facility must comply with the following additional requirements—
- (i) All drugs and biologicals must be stored in secure areas. All controlled drugs listed in Schedules II, III, IV, and V of the Comprehensive Drug Abuse Prevention and Control Act of 1976 must be stored in locked compartments within such secure storage areas. Only personnel authorized to administer controlled drugs as noted in paragraph (d)(2) of this section may have access to the locked compartments; and
  - (ii) Discrepancies in the acquisition, storage, dispensing, administration, disposal, or return of controlled drugs must be investigated immediately by the pharmacist and hospice administrator and where required reported to the appropriate State authority. A written account of the investigation must be made available to State and Federal officials if required by law or regulation.

(f) **Standard: Use and maintenance of equipment and supplies.**

- (1) The hospice must ensure that manufacturer recommendations for performing routine and preventive maintenance on durable medical equipment are followed. The equipment must be safe and work as intended for use in the patient's environment. Where a manufacturer recommendation for a piece of equipment does not exist, the hospice must ensure that repair and routine maintenance policies are developed. The hospice may use persons under contract to ensure the maintenance and repair of durable medical equipment.
- (2) The hospice must ensure that the patient, where appropriate, as well as the family and/or other caregiver(s), receive instruction in the safe use of durable medical equipment and supplies. The hospice may use persons under contract to ensure patient and family instruction. The patient, family, and/or caregiver must be able to demonstrate the appropriate use of durable medical equipment to the satisfaction of the hospice staff.
- (3) Hospices may only contract for durable medical equipment services with a durable medical equipment supplier that meets the Medicare DMEPOS Supplier Quality and Accreditation Standards at 42 CFR 424.57.

[73 FR 32204, June 5, 2008, as amended at 84 FR 51815, Sept. 30, 2019; 84 FR 63202, Nov. 15, 2019]

**§ 418.108 Condition of participation: Short-term inpatient care.**

Inpatient care must be available for pain control, symptom management, and respite purposes, and must be provided in a participating Medicare or Medicaid facility.

(a) **Standard: Inpatient care for symptom management and pain control.** Inpatient care for pain control and symptom management must be provided in one of the following:

- (1) A Medicare-certified hospice that meets the conditions of participation for providing inpatient care directly as specified in § 418.110.
- (2) A Medicare-certified hospital or a skilled nursing facility that also meets the standards specified in § 418.110(b) and (f) regarding 24-hour nursing services and patient areas.

(b) **Standard: Inpatient care for respite purposes.**

- (1) Inpatient care for respite purposes must be provided by one of the following:

- (i) A provider specified in paragraph (a) of this section.
  - (ii) A Medicare or Medicaid-certified nursing facility that also meets the standards specified in § 418.110(f).
- (2) The facility providing respite care must provide 24-hour nursing services that meet the nursing needs of all patients and are furnished in accordance with each patient's plan of care. Each patient must receive all nursing services as prescribed and must be kept comfortable, clean, well-groomed, and protected from accident, injury, and infection.
- (c) **Standard: Inpatient care provided under arrangements.** If the hospice has an arrangement with a facility to provide for short-term inpatient care, the arrangement is described in a written agreement, coordinated by the hospice, and at a minimum specifies—
  - (1) That the hospice supplies the inpatient provider a copy of the patient's plan of care and specifies the inpatient services to be furnished;
  - (2) That the inpatient provider has established patient care policies consistent with those of the hospice and agrees to abide by the palliative care protocols and plan of care established by the hospice for its patients;
  - (3) That the hospice patient's inpatient clinical record includes a record of all inpatient services furnished and events regarding care that occurred at the facility; that a copy of the discharge summary be provided to the hospice at the time of discharge; and that a copy of the inpatient clinical record is available to the hospice at the time of discharge;
  - (4) That the inpatient facility has identified an individual within the facility who is responsible for the implementation of the provisions of the agreement;
  - (5) That the hospice retains responsibility for ensuring that the training of personnel who will be providing the patient's care in the inpatient facility has been provided and that a description of the training and the names of those giving the training are documented; and
  - (6) A method for verifying that the requirements in paragraphs (c)(1) through (c)(5) of this section are met.
- (d) **Standard: Inpatient care limitation.** The total number of inpatient days used by Medicare beneficiaries who elected hospice coverage in a 12-month period in a particular hospice may not exceed 20 percent of the total number of hospice days consumed in total by this group of beneficiaries.
- (e) **Standard: Exemption from limitation.** Before October 1, 1986, any hospice that began operation before January 1, 1975, is not subject to the limitation specified in paragraph (d) of this section.

[73 FR 32204, June 5, 2008, as amended at 74 FR 39413, Aug. 6, 2009; 81 FR 26897, May 4, 2016]

#### **§ 418.110 Condition of participation: Hospices that provide inpatient care directly.**

A hospice that provides inpatient care directly in its own facility must demonstrate compliance with all of the following standards:

- (a) **Standard: Staffing.** The hospice is responsible for ensuring that staffing for all services reflects its volume of patients, their acuity, and the level of intensity of services needed to ensure that plan of care outcomes are achieved and negative outcomes are avoided.

(b) **Standard: Twenty-four hour nursing services.**

- (1) The hospice facility must provide 24-hour nursing services that meet the nursing needs of all patients and are furnished in accordance with each patient's plan of care. Each patient must receive all nursing services as prescribed and must be kept comfortable, clean, well-groomed, and protected from accident, injury, and infection.
- (2) If at least one patient in the hospice facility is receiving general inpatient care, then each shift must include a registered nurse who provides direct patient care.

(c) **Standard: Physical environment.** The hospice must maintain a safe physical environment free of hazards for patients, staff, and visitors.

- (1) **Safety management.** The hospice must address real or potential threats to the health and safety of the patients, others, and property.
- (2) **Physical plant and equipment.** The hospice must develop procedures for controlling the reliability and quality of—
  - (i) The routine storage and prompt disposal of trash and medical waste;
  - (ii) Light, temperature, and ventilation/air exchanges throughout the hospice;
  - (iii) Emergency gas and water supply; and
  - (iv) The scheduled and emergency maintenance and repair of all equipment.

(d) **Standard: Fire protection.**

- (1) Except as otherwise provided in this section—
  - (i) The hospice must meet the applicable provisions and must proceed in accordance with the Life Safety Code (NFPA 101 and Tentative Interim Amendments TIA 12-1, TIA 12-2, TIA 12-3, and TIA 12-4.)
  - (ii) Notwithstanding paragraph (d)(1)(i) of this section, corridor doors and doors to rooms containing flammable or combustible materials must be provided with positive latching hardware. Roller latches are prohibited on such doors.
- (2) In consideration of a recommendation by the State survey agency or Accrediting Organization or at the discretion of the Secretary, may waive, for periods deemed appropriate, specific provisions of the Life Safety Code, which would result in unreasonable hardship upon a hospice facility, but only if the waiver will not adversely affect the health and safety of the patients.
- (3) The provisions of the adopted edition of the Life Safety Code do not apply in a State if CMS finds that a fire and safety code imposed by State law adequately protects patients in hospices.
- (4) A hospice may place alcohol-based hand rub dispensers in its facility if the dispensers are installed in a manner that adequately protects against access by vulnerable populations.
- (5) When a sprinkler system is shut down for more than 10 hours, the hospice must:
  - (i) Evacuate the building or portion of the building affected by the system outage until the system is back in service, or
  - (ii) Establish a fire watch until the system is back in service.

- (6) Buildings must have an outside window or outside door in every sleeping room, and for any building constructed after July 5, 2016 the sill height must not exceed 36 inches above the floor. Windows in atrium walls are considered outside windows for the purposes of this requirement.
- (e) **Standard: Building Safety.** Except as otherwise provided in this section, the hospice must meet the applicable provisions and must proceed in accordance with the Health Care Facilities Code (NFPA 99 and Tentative Interim Amendments TIA 12-2, TIA 12-3, TIA 12-4, TIA 12-5 and TIA 12-6).
  - (1) Chapters 7, 8, 12, and 13 of the adopted Health Care Facilities Code do not apply to a hospice.
  - (2) If application of the Health Care Facilities Code required under paragraph (e) of this section would result in unreasonable hardship for the hospice, CMS may waive specific provisions of the Health Care Facilities Code, but only if the waiver does not adversely affect the health and safety of patients.
- (f) **Standard: Patient areas.** The hospice must provide a home-like atmosphere and ensure that patient areas are designed to preserve the dignity, comfort, and privacy of patients.
  - (1) The hospice must provide—
    - (i) Physical space for private patient and family visiting;
    - (ii) Accommodations for family members to remain with the patient throughout the night; and
    - (iii) Physical space for family privacy after a patient's death.
  - (2) The hospice must provide the opportunity for patients to receive visitors at any hour, including infants and small children.
- (g) **Standard: Patient rooms.**
  - (1) The hospice must ensure that patient rooms are designed and equipped for nursing care, as well as the dignity, comfort, and privacy of patients.
  - (2) The hospice must accommodate a patient and family request for a single room whenever possible.
  - (3) Each patient's room must—
    - (i) Be at or above grade level;
    - (ii) Contain a suitable bed and other appropriate furniture for each patient;
    - (iii) Have closet space that provides security and privacy for clothing and personal belongings;
    - (iv) Accommodate no more than two patients and their family members;
    - (v) Provide at least 80 square feet for each residing patient in a double room and at least 100 square feet for each patient residing in a single room; and
    - (vi) Be equipped with an easily-activated, functioning device accessible to the patient, that is used for calling for assistance.
  - (4) For a facility occupied by a Medicare-participating hospice on December 2, 2008, CMS may waive the space and occupancy requirements of paragraphs (g)(2)(iv) and (g)(2)(v) of this section if it determines that—
    - (i) Imposition of the requirements would result in unreasonable hardship on the hospice if strictly enforced; or jeopardize its ability to continue to participate in the Medicare program; and

- (ii) The waiver serves the needs of the patient and does not adversely affect their health and safety.
- (h) **Standard: Toilet and bathing facilities.** Each patient room must be equipped with, or conveniently located near, toilet and bathing facilities.
- (i) **Standard: Plumbing facilities.** The hospice must—
  - (1) Have an adequate supply of hot water at all times; and
  - (2) Have plumbing fixtures with control valves that automatically regulate the temperature of the hot water used by patients.
- (j) **Standard: Infection control.** The hospice must maintain an infection control program that protects patients, staff and others by preventing and controlling infections and communicable disease as stipulated in § 418.60.
- (k) **Standard: Sanitary environment.** The hospice must provide a sanitary environment by following current standards of practice, including nationally recognized infection control precautions, and avoid sources and transmission of infections and communicable diseases.
- (l) **Standard: Linen.** The hospice must have available at all times a quantity of clean linen in sufficient amounts for all patient uses. Linens must be handled, stored, processed, and transported in such a manner as to prevent the spread of contaminants.
- (m) **Standard: Meal service and menu planning.** The hospice must furnish meals to each patient that are—
  - (1) Consistent with the patient's plan of care, nutritional needs, and therapeutic diet;
  - (2) Palatable, attractive, and served at the proper temperature; and
  - (3) Obtained, stored, prepared, distributed, and served under sanitary conditions.
- (n) **Standard: Restraint or seclusion.** All patients have the right to be free from physical or mental abuse, and corporal punishment. All patients have the right to be free from restraint or seclusion, of any form, imposed as a means of coercion, discipline, convenience, or retaliation by staff. Restraint or seclusion may only be imposed to ensure the immediate physical safety of the patient, a staff member, or others and must be discontinued at the earliest possible time.
  - (1) Restraint or seclusion may only be used when less restrictive interventions have been determined to be ineffective to protect the patient, a staff member, or others from harm.
  - (2) The type or technique of restraint or seclusion used must be the least restrictive intervention that will be effective to protect the patient, a staff member, or others from harm.
  - (3) The use of restraint or seclusion must be—
    - (i) In accordance with a written modification to the patient's plan of care; and
    - (ii) Implemented in accordance with safe and appropriate restraint and seclusion techniques as determined by hospice policy in accordance with State law.
  - (4) The use of restraint or seclusion must be in accordance with the order of a physician authorized to order restraint or seclusion by hospice policy in accordance with State law.
  - (5) Orders for the use of restraint or seclusion must never be written as a standing order or on an as needed basis (PRN).

- (6) The medical director or physician designee must be consulted as soon as possible if the attending physician did not order the restraint or seclusion.
- (7) Unless superseded by State law that is more restrictive—
  - (i) Each order for restraint or seclusion used for the management of violent or self-destructive behavior that jeopardizes the immediate physical safety of the patient, a staff member, or others may only be renewed in accordance with the following limits for up to a total of 24 hours:
    - (A) 4 hours for adults 18 years of age or older;
    - (B) 2 hours for children and adolescents 9 to 17 years of age; or
    - (C) 1 hour for children under 9 years of age; and

After 24 hours, before writing a new order for the use of restraint or seclusion for the management of violent or self-destructive behavior, a physician authorized to order restraint or seclusion by hospice policy in accordance with State law must see and assess the patient.
  - (ii) Each order for restraint used to ensure the physical safety of the non-violent or non-self-destructive patient may be renewed as authorized by hospice policy.
- (8) Restraint or seclusion must be discontinued at the earliest possible time, regardless of the length of time identified in the order.
- (9) The condition of the patient who is restrained or secluded must be monitored by a physician or trained staff that have completed the training criteria specified in paragraph (o) of this section at an interval determined by hospice policy.
- (10) Physician, including attending physician, training requirements must be specified in hospice policy. At a minimum, physicians and attending physicians authorized to order restraint or seclusion by hospice policy in accordance with State law must have a working knowledge of hospice policy regarding the use of restraint or seclusion.
- (11) When restraint or seclusion is used for the management of violent or self-destructive behavior that jeopardizes the immediate physical safety of the patient, a staff member, or others, the patient must be seen face-to-face within 1 hour after the initiation of the intervention—
  - (i) By a—
    - (A) Physician; or
    - (B) Registered nurse who has been trained in accordance with the requirements specified in paragraph (n) of this section.
  - (ii) To evaluate—
    - (A) The patient's immediate situation;
    - (B) The patient's reaction to the intervention;
    - (C) The patient's medical and behavioral condition; and
    - (D) The need to continue or terminate the restraint or seclusion.



- (12) States are free to have requirements by statute or regulation that are more restrictive than those contained in paragraph (m)(11)(i) of this section.
- (13) If the face-to-face evaluation specified in § 418.110(n)(11) is conducted by a trained registered nurse, the trained registered nurse must consult the medical director or physician designee as soon as possible after the completion of the 1-hour face-to-face evaluation.
- (14) All requirements specified under this paragraph are applicable to the simultaneous use of restraint and seclusion. Simultaneous restraint and seclusion use is only permitted if the patient is continually monitored—
  - (i) Face-to-face by an assigned, trained staff member; or
  - (ii) By trained staff using both video and audio equipment. This monitoring must be in close proximity to the patient.
- (15) When restraint or seclusion is used, there must be documentation in the patient's clinical record of the following:
  - (i) The 1-hour face-to-face medical and behavioral evaluation if restraint or seclusion is used to manage violent or self-destructive behavior;
  - (ii) A description of the patient's behavior and the intervention used;
  - (iii) Alternatives or other less restrictive interventions attempted (as applicable);
  - (iv) The patient's condition or symptom(s) that warranted the use of the restraint or seclusion; and the patient's response to the intervention(s) used, including the rationale for continued use of the intervention.
- (o) **Standard: Restraint or seclusion staff training requirements.** The patient has the right to safe implementation of restraint or seclusion by trained staff.
  - (1) **Training intervals.** All patient care staff working in the hospice inpatient facility must be trained and able to demonstrate competency in the application of restraints, implementation of seclusion, monitoring, assessment, and providing care for a patient in restraint or seclusion—
    - (i) Before performing any of the actions specified in this paragraph;
    - (ii) As part of orientation; and
    - (iii) Subsequently on a periodic basis consistent with hospice policy.
  - (2) **Training content.** The hospice must require appropriate staff to have education, training, and demonstrated knowledge based on the specific needs of the patient population in at least the following:
    - (i) Techniques to identify staff and patient behaviors, events, and environmental factors that may trigger circumstances that require the use of a restraint or seclusion.
    - (ii) The use of nonphysical intervention skills.
    - (iii) Choosing the least restrictive intervention based on an individualized assessment of the patient's medical, or behavioral status or condition.

- (iv) The safe application and use of all types of restraint or seclusion used in the hospice, including training in how to recognize and respond to signs of physical and psychological distress (for example, positional asphyxia).
  - (v) Clinical identification of specific behavioral changes that indicate that restraint or seclusion is no longer necessary.
  - (vi) Monitoring the physical and psychological well-being of the patient who is restrained or secluded, including but not limited to, respiratory and circulatory status, skin integrity, vital signs, and any special requirements specified by hospice policy associated with the 1-hour face-to-face evaluation.
  - (vii) The use of first aid techniques and certification in the use of cardiopulmonary resuscitation, including required periodic recertification.
- (3) **Trainer requirements.** Individuals providing staff training must be qualified as evidenced by education, training, and experience in techniques used to address patients' behaviors.
- (4) **Training documentation.** The hospice must document in the staff personnel records that the training and demonstration of competency were successfully completed.
- (p) **Standard: Death reporting requirements.** Hospices must report deaths associated with the use of seclusion or restraint.
- (1) The hospice must report the following information to CMS:
    - (i) Each unexpected death that occurs while a patient is in restraint or seclusion.
    - (ii) Each unexpected death that occurs within 24 hours after the patient has been removed from restraint or seclusion.
    - (iii) Each death known to the hospice that occurs within 1 week after restraint or seclusion where it is reasonable to assume that use of restraint or placement in seclusion contributed directly or indirectly to a patient's death. "Reasonable to assume" in this context includes, but is not limited to, deaths related to restrictions of movement for prolonged periods of time, or death related to chest compression, restriction of breathing or asphyxiation.
  - (2) Each death referenced in this paragraph must be reported to CMS by telephone no later than the close of business the next business day following knowledge of the patient's death.
  - (3) Staff must document in the patient's clinical record the date and time the death was reported to CMS.
- (q) The standards incorporated by reference in this section are approved for incorporation by reference by the Director of the Office of the Federal Register in accordance with 5 U.S.C. 552(a) and 1 CFR part 51. You may inspect a copy at the CMS Information Resource Center, 7500 Security Boulevard, Baltimore, MD or at the National Archives and Records Administration (NARA). For information on the availability of this material at NARA, call 202-741-6030, or go to: [http://www.archives.gov/federal\\_register/code\\_of\\_federal\\_regulations/ibr\\_locations.html](http://www.archives.gov/federal_register/code_of_federal_regulations/ibr_locations.html). If any changes in this edition of the Code are incorporated by reference, CMS will publish a document in the FEDERAL REGISTER to announce the changes.
- (1) National Fire Protection Association, 1 Batterymarch Park, Quincy, MA 02169, [www.nfpa.org](http://www.nfpa.org), 1.617.770.3000.

- (i) NFPA 99, Standards for Health Care Facilities Code of the National Fire Protection Association 99, 2012 edition, issued August 11, 2011.
- (ii) TIA 12-2 to NFPA 99, issued August 11, 2011.
- (iii) TIA 12-3 to NFPA 99, issued August 9, 2012.
- (iv) TIA 12-4 to NFPA 99, issued March 7, 2013.
- (v) TIA 12-5 to NFPA 99, issued August 1, 2013.
- (vi) TIA 12-6 to NFPA 99, issued March 3, 2014.
- (vii) NFPA 101, Life Safety Code, 2012 edition, issued August 11, 2011;
- (viii) TIA 12-1 to NFPA 101, issued August 11, 2011.
- (ix) TIA 12-2 to NFPA 101, issued October 30, 2012.
- (x) TIA 12-3 to NFPA 101, issued October 22, 2013.
- (xi) TIA 12-4 to NFPA 101, issued October 22, 2013.
- (2) [Reserved]

[73 FR 32204, June 5, 2008, as amended at 81 FR 26879, May 4, 2016; 81 FR 64024, Sept. 16, 2016]

**§ 418.112 Condition of participation: Hospices that provide hospice care to residents of a SNF/NF or ICF/IID.**

In addition to meeting the conditions of participation at § 418.10 through § 418.116, a hospice that provides hospice care to residents of a SNF/NF or ICF/IID must abide by the following additional standards.

- (a) **Standard: Resident eligibility, election, and duration of benefits.** Medicare patients receiving hospice services and residing in a SNF, NF, or ICF/IID are subject to the Medicare hospice eligibility criteria set out at § 418.20 through § 418.30.
- (b) **Standard: Professional management.** The hospice must assume responsibility for professional management of the resident's hospice services provided, in accordance with the hospice plan of care and the hospice conditions of participation, and make any arrangements necessary for hospice-related inpatient care in a participating Medicare/Medicaid facility according to §§ 418.100 and 418.108.
- (c) **Standard: Written agreement.** The hospice and SNF/NF or ICF/IID must have a written agreement that specifies the provision of hospice services in the facility. The agreement must be signed by authorized representatives of the hospice and the SNF/NF or ICF/IID before the provision of hospice services. The written agreement must include at least the following:
  - (1) The manner in which the SNF/NF or ICF/IID and the hospice are to communicate with each other and document such communications to ensure that the needs of patients are addressed and met 24 hours a day.
  - (2) A provision that the SNF/NF or ICF/IID immediately notifies the hospice if—
    - (i) A significant change in a patient's physical, mental, social, or emotional status occurs;
    - (ii) Clinical complications appear that suggest a need to alter the plan of care;

- (iii) A need to transfer a patient from the SNF/NF or ICF/IID, and the hospice makes arrangements for, and remains responsible for, any necessary continuous care or inpatient care necessary related to the terminal illness and related conditions; or
  - (iv) A patient dies.
- (3) A provision stating that the hospice assumes responsibility for determining the appropriate course of hospice care, including the determination to change the level of services provided.
  - (4) An agreement that it is the SNF/NF or ICF/IID responsibility to continue to furnish 24 hour room and board care, meeting the personal care and nursing needs that would have been provided by the primary caregiver at home at the same level of care provided before hospice care was elected.
  - (5) An agreement that it is the hospice's responsibility to provide services at the same level and to the same extent as those services would be provided if the SNF/NF or ICF/IID resident were in his or her own home.
  - (6) A delineation of the hospice's responsibilities, which include, but are not limited to the following: Providing medical direction and management of the patient; nursing; counseling (including spiritual, dietary and bereavement); social work; provision of medical supplies, durable medical equipment and drugs necessary for the palliation of pain and symptoms associated with the terminal illness and related conditions; and all other hospice services that are necessary for the care of the resident's terminal illness and related conditions.
  - (7) A provision that the hospice may use the SNF/NF or ICF/IID nursing personnel where permitted by State law and as specified by the SNF/NF or ICF/IID to assist in the administration of prescribed therapies included in the plan of care only to the extent that the hospice would routinely use the services of a hospice patient's family in implementing the plan of care.
  - (8) A provision stating that the hospice must report all alleged violations involving mistreatment, neglect, or verbal, mental, sexual, and physical abuse, including injuries of unknown source, and misappropriation of patient property by anyone unrelated to the hospice to the SNF/NF or ICF/IID administrator within 24 hours of the hospice becoming aware of the alleged violation.
  - (9) A delineation of the responsibilities of the hospice and the SNF/NF or ICF/IID to provide bereavement services to SNF/NF or ICF/IID staff.
- (d) **Standard: Hospice plan of care.** In accordance with § 418.56, a written hospice plan of care must be established and maintained in consultation with SNF/NF or ICF/IID representatives. All hospice care provided must be in accordance with this hospice plan of care.
    - (1) The hospice plan of care must identify the care and services that are needed and specifically identify which provider is responsible for performing the respective functions that have been agreed upon and included in the hospice plan of care.
    - (2) The hospice plan of care reflects the participation of the hospice, the SNF/NF or ICF/IID, and the patient and family to the extent possible.
    - (3) Any changes in the hospice plan of care must be discussed with the patient or representative, and SNF/NF or ICF/IID representatives, and must be approved by the hospice before implementation.
  - (e) **Standard: Coordination of services.** The hospice must:

- (1) Designate a member of each interdisciplinary group that is responsible for a patient who is a resident of a SNF/NF or ICF/IID. The designated interdisciplinary group member is responsible for:
  - (i) Providing overall coordination of the hospice care of the SNF/NF or ICF/IID resident with SNF/NF or ICF/IID representatives; and
  - (ii) Communicating with SNF/NF or ICF/IID representatives and other health care providers participating in the provision of care for the terminal illness and related conditions and other conditions to ensure quality of care for the patient and family.
- (2) Ensure that the hospice IDG communicates with the SNF/NF or ICF/IID medical director, the patient's attending physician, and other physicians participating in the provision of care to the patient as needed to coordinate the hospice care of the hospice patient with the medical care provided by other physicians.
- (3) Provide the SNF/NF or ICF/IID with the following information:
  - (i) The most recent hospice plan of care specific to each patient;
  - (ii) Hospice election form and any advance directives specific to each patient;
  - (iii) Physician certification and recertification of the terminal illness specific to each patient;
  - (iv) Names and contact information for hospice personnel involved in hospice care of each patient;
  - (v) Instructions on how to access the hospice's 24-hour on-call system;
  - (vi) Hospice medication information specific to each patient; and
  - (vii) Hospice physician and attending physician (if any) orders specific to each patient.
- (f) **Standard: Orientation and training of staff.** Hospice staff, in coordination with SNF/NF or ICF/IID facility staff, must assure orientation of such staff furnishing care to hospice patients in the hospice philosophy, including hospice policies and procedures regarding methods of comfort, pain control, symptom management, as well as principles about death and dying, individual responses to death, patient rights, appropriate forms, and record keeping requirements.

[73 FR 32204, June 5, 2008, as amended at 84 FR 51815, Sept. 30, 2019]

### § 418.113 Condition of participation: Emergency preparedness.

The hospice must comply with all applicable Federal, State, and local emergency preparedness requirements. The hospice must establish and maintain an emergency preparedness program that meets the requirements of this section. The emergency preparedness program must include, but not be limited to, the following elements:

- (a) **Emergency plan.** The hospice must develop and maintain an emergency preparedness plan that must be reviewed, and updated at least every 2 years. The plan must do the following:
  - (1) Be based on and include a documented, facility-based and community-based risk assessment, utilizing an all-hazards approach.
  - (2) Include strategies for addressing emergency events identified by the risk assessment, including the management of the consequences of power failures, natural disasters, and other emergencies that would affect the hospice's ability to provide care.

- (3) Address patient population, including, but not limited to, the type of services the hospice has the ability to provide in an emergency; and continuity of operations, including delegations of authority and succession plans.
- (4) Include a process for cooperation and collaboration with local, tribal, regional, State, or Federal emergency preparedness officials' efforts to maintain an integrated response during a disaster or emergency situation.

(b) **Policies and procedures.** The hospice must develop and implement emergency preparedness policies and procedures, based on the emergency plan set forth in paragraph (a) of this section, risk assessment at paragraph (a)(1) of this section, and the communication plan at paragraph (c) of this section. The policies and procedures must be reviewed and updated at least every 2 years. At a minimum, the policies and procedures must address the following:

- (1) Procedures to follow up with on-duty staff and patients to determine services that are needed, in the event that there is an interruption in services during or due to an emergency. The hospice must inform State and local officials of any on-duty staff or patients that they are unable to contact.
- (2) Procedures to inform State and local officials about hospice patients in need of evacuation from their residences at any time due to an emergency situation based on the patient's medical and psychiatric condition and home environment.
- (3) A system of medical documentation that preserves patient information, protects confidentiality of patient information, and secures and maintains the availability of records.
- (4) The use of hospice employees in an emergency and other emergency staffing strategies, including the process and role for integration of State and Federally designated health care professionals to address surge needs during an emergency.
- (5) The development of arrangements with other hospices and other providers to receive patients in the event of limitations or cessation of operations to maintain the continuity of services to hospice patients.
- (6) The following are additional requirements for hospice-operated inpatient care facilities only. The policies and procedures must address the following:
  - (i) A means to shelter in place for patients, hospice employees who remain in the hospice.
  - (ii) Safe evacuation from the hospice, which includes consideration of care and treatment needs of evacuees; staff responsibilities; transportation; identification of evacuation location(s) and primary and alternate means of communication with external sources of assistance.
  - (iii) The provision of subsistence needs for hospice employees and patients, whether they evacuate or shelter in place, include, but are not limited to the following:
    - (A) Food, water, medical, and pharmaceutical supplies.
    - (B) Alternate sources of energy to maintain the following:
      - (1) Temperatures to protect patient health and safety and for the safe and sanitary storage of provisions.
      - (2) Emergency lighting.
      - (3) Fire detection, extinguishing, and alarm systems.

- (C) Sewage and waste disposal.
  - (iv) The role of the hospice under a waiver declared by the Secretary, in accordance with section 1135 of the Act, in the provision of care and treatment at an alternate care site identified by emergency management officials.
  - (v) A system to track the location of hospice employees' on-duty and sheltered patients in the hospice's care during an emergency. If the on-duty employees or sheltered patients are relocated during the emergency, the hospice must document the specific name and location of the receiving facility or other location.
- (c) **Communication plan.** The hospice must develop and maintain an emergency preparedness communication plan that complies with Federal, State, and local laws and must be reviewed and updated at least every 2 years. The communication plan must include all of the following:
  - (1) Names and contact information for the following:
    - (i) Hospice employees.
    - (ii) Entities providing services under arrangement.
    - (iii) Patients' physicians.
    - (iv) Other hospices.
  - (2) Contact information for the following:
    - (i) Federal, State, tribal, regional, and local emergency preparedness staff.
    - (ii) Other sources of assistance.
  - (3) Primary and alternate means for communicating with the following:
    - (i) Hospice's employees.
    - (ii) Federal, State, tribal, regional, and local emergency management agencies.
  - (4) A method for sharing information and medical documentation for patients under the hospice's care, as necessary, with other health care providers to maintain the continuity of care.
  - (5) A means, in the event of an evacuation, to release patient information as permitted under 45 CFR 164.510(b)(1)(ii).
  - (6) A means of providing information about the general condition and location of patients under the facility's care as permitted under 45 CFR 164.510(b)(4).
  - (7) A means of providing information about the hospice's inpatient occupancy, needs, and its ability to provide assistance, to the authority having jurisdiction, the Incident Command Center, or designee.
- (d) **Training and testing.** The hospice must develop and maintain an emergency preparedness training and testing program that is based on the emergency plan set forth in paragraph (a) of this section, risk assessment at paragraph (a)(1) of this section, policies and procedures at paragraph (b) of this section, and the communication plan at paragraph (c) of this section. The training and testing program must be reviewed and updated at least every 2 years.
  - (1) **Training program.** The hospice must do all of the following:

- (i) Initial training in emergency preparedness policies and procedures to all new and existing hospice employees, and individuals providing services under arrangement, consistent with their expected roles.
  - (ii) Demonstrate staff knowledge of emergency procedures.
  - (iii) Provide emergency preparedness training at least every 2 years.
  - (iv) Periodically review and rehearse its emergency preparedness plan with hospice employees (including nonemployee staff), with special emphasis placed on carrying out the procedures necessary to protect patients and others.
  - (v) Maintain documentation of all emergency preparedness training.
  - (vi) If the emergency preparedness policies and procedures are significantly updated, the hospice must conduct training on the updated policies and procedures.
- (2) **Testing for hospices that provide care in the patient's home.** The hospice must conduct exercises to test the emergency plan at least annually. The hospice must do the following:
- (i) Participate in a full-scale exercise that is community-based every 2 years; or
    - (A) When a community-based exercise is not accessible, conduct an individual facility-based functional exercise every 2 years; or
    - (B) If the hospice experiences a natural or man-made emergency that requires activation of the emergency plan, the hospital is exempt from engaging in its next required full-scale community-based exercise or individual facility-based functional exercise following the onset of the emergency event.
  - (ii) Conduct an additional exercise every 2 years, opposite the year the full-scale or functional exercise under paragraph (d)(2)(i) of this section is conducted, that may include, but is not limited to the following:
    - (A) A second full-scale exercise that is community-based or a facility-based functional exercise; or
    - (B) A mock disaster drill; or
    - (C) A tabletop exercise or workshop that is led by a facilitator and includes a group discussion using a narrated, clinically-relevant emergency scenario, and a set of problem statements, directed messages, or prepared questions designed to challenge an emergency plan.
- (3) **Testing for hospices that provide inpatient care directly.** The hospice must conduct exercises to test the emergency plan twice per year. The hospice must do the following:
- (i) Participate in an annual full-scale exercise that is community-based; or
    - (A) When a community-based exercise is not accessible, conduct an annual individual facility-based functional exercise; or
    - (B) If the hospice experiences a natural or man-made emergency that requires activation of the emergency plan, the hospice is exempt from engaging in its next required full-scale community-based or facility-based functional exercise following the onset of the emergency event.



- (ii) Conduct an additional annual exercise that may include, but is not limited to the following:
    - (A) A second full-scale exercise that is community-based or a facility-based functional exercise; or
    - (B) A mock disaster drill; or
    - (C) A tabletop exercise or workshop led by a facilitator that includes a group discussion using a narrated, clinically-relevant emergency scenario, and a set of problem statements, directed messages, or prepared questions designed to challenge an emergency plan.
  - (iii) Analyze the hospice's response to and maintain documentation of all drills, tabletop exercises, and emergency events and revise the hospice's emergency plan, as needed.
- (e) **Integrated healthcare systems.** If a hospice is part of a healthcare system consisting of multiple separately certified healthcare facilities that elects to have a unified and integrated emergency preparedness program, the hospice may choose to participate in the healthcare system's coordinated emergency preparedness program. If elected, the unified and integrated emergency preparedness program must do the following:
- (1) Demonstrate that each separately certified facility within the system actively participated in the development of the unified and integrated emergency preparedness program.
  - (2) Be developed and maintained in a manner that takes into account each separately certified facility's unique circumstances, patient populations, and services offered.
  - (3) Demonstrate that each separately certified facility is capable of actively using the unified and integrated emergency preparedness program and is in compliance with the program.
  - (4) Include a unified and integrated emergency plan that meets the requirements of paragraphs (a)(2), (3), and (4) of this section. The unified and integrated emergency plan must also be based on and include the following:
    - (i) A documented community-based risk assessment, utilizing an all-hazards approach.
    - (ii) A documented individual facility-based risk assessment for each separately certified facility within the health system, utilizing an all-hazards approach.
  - (5) Include integrated policies and procedures that meet the requirements set forth in paragraph (b) of this section, a coordinated communication plan and training and testing programs that meet the requirements of paragraphs (c) and (d) of this section, respectively.

[81 FR 64024, Sept. 16, 2016, as amended at 84 FR 51815, Sept. 30, 2019]

#### § 418.114 Condition of participation: Personnel qualifications.

- (a) **General qualification requirements.** Except as specified in paragraph (c) of this section, all professionals who furnish services directly, under an individual contract, or under arrangements with a hospice, must be legally authorized (licensed, certified or registered) in accordance with applicable Federal, State and local laws, and must act only within the scope of his or her State license, or State certification, or registration. All personnel qualifications must be kept current at all times.
- (b) **Personnel qualifications for certain disciplines.** The following qualifications must be met:

- (1) **Physician.** Physicians must meet the qualifications and conditions as defined in section 1861(r) of the Act and implemented at § 410.20 of this chapter.
- (2) **Hospice aide.** Hospice aides must meet the qualifications required by section 1891(a)(3) of the Act and implemented at § 418.76.
- (3) **Social worker.** A person who—
  - (i)
    - (A) Has a Master of Social Work (MSW) degree from a school of social work accredited by the Council on Social Work Education; or
    - (B) Has a baccalaureate degree in social work from an institution accredited by the Council on Social Work Education; or a baccalaureate degree in psychology, sociology, or other field related to social work and is supervised by an MSW as described in paragraph (b)(3)(i)(A) of this section; and
  - (ii) Has 1 year of social work experience in a healthcare setting; or
  - (iii) Has a baccalaureate degree from a school of social work accredited by the Council on Social Work Education, is employed by the hospice before December 2, 2008, and is not required to be supervised by an MSW.
- (4) **Speech language pathologist.** A person who meets either of the following requirements:
  - (i) The education and experience requirements for a Certificate of Clinical Competence in speech-language pathology granted by the American Speech-Language-Hearing Association.
  - (ii) The educational requirements for certification and is in the process of accumulating the supervised experience required for certification.
- (5) **Occupational therapist.** A person who—
  - (i)
    - (A) Is licensed or otherwise regulated, if applicable, as an occupational therapist by the State in which practicing, unless licensure does not apply;
    - (B) Graduated after successful completion of an occupational therapist education program accredited by the Accreditation Council for Occupational Therapy Education (ACOTE) of the American Occupational Therapy Association, Inc. (AOTA), or successor organizations of ACOTE; and
    - (C) Is eligible to take, or has successfully completed the entry-level certification examination for occupational therapists developed and administered by the National Board for Certification in Occupational Therapy, Inc. (NBCOT).
  - (ii) On or before December 31, 2009—
    - (A) Is licensed or otherwise regulated, if applicable, as an occupational therapist by the State in which practicing; or
    - (B) When licensure or other regulation does not apply—

- (1) Graduated after successful completion of an occupational therapist education program accredited by the accreditation Council for Occupational therapy Education (ACOTE) of the American Occupational Therapy Association, Inc. (AOTA) or successor organizations of ACOTE; and
    - (2) Is eligible to take, or has successfully completed the entry-level certification examination for occupational therapists developed and administered by the National Board for Certification in Occupational Therapy, Inc., (NBCOT).
  - (iii) On or before January 1, 2008—
    - (A) Graduated after successful completion of an occupational therapy program accredited jointly by the committee on Allied Health Education and Accreditation of the American Medical Association and the American Occupational Therapy Association; or
    - (B) Is eligible for the National Registration Examination of the American Occupational Therapy Association or the National Board for Certification in Occupational Therapy.
  - (iv) On or before December 31, 1977—
    - (A) Had 2 years of appropriate experience as an occupational therapist; and
    - (B) Had achieved a satisfactory grade on an occupational therapist proficiency examination conducted, approved, or sponsored by the U.S. Public Health Service.
  - (v) If educated outside the United States—
    - (A) Must meet both of the following:
      - (1) Graduated after successful completion of an occupational therapist education program accredited as substantially equivalent to occupational therapist assistant entry level education in the United States by one of the following:
        - (i) The Accreditation Council for Occupational Therapy Education (ACOTE).
        - (ii) Successor organizations of ACOTE.
        - (iii) The World Federation of Occupational Therapists.
        - (iv) A credentialing body approved by the American Occupational Therapy Association.
        - (v) Successfully completed the entry level certification examination for occupational therapists developed and administered by the National Board for Certification in Occupational Therapy, Inc. (NBCOT).
      - (2) On or before December 31, 2009, is licensed or otherwise regulated, if applicable, as an occupational therapist by the State in which practicing.
- (6) **Occupational therapy assistant.** A person who
  - (i) Meets all of the following:
    - (A) Is licensed or otherwise regulated, if applicable, as an occupational therapy assistant by the State in which practicing, unless licensure does apply.

- (B) Graduated after successful completion of an occupational therapy assistant education program accredited by the Accreditation Council for Occupational Therapy Education (ACOTE) of the American Occupational Therapy Association, Inc. (AOTA) or its successor organizations.
  - (C) Is eligible to take or successfully completed the entry-level certification examination for occupational therapy assistants developed and administered by the National Board for Certification in Occupational Therapy, Inc. (NBCOT).
- (ii) On or before December 31, 2009—
  - (A) Is licensed or otherwise regulated as an occupational therapy assistant, if applicable, by the State in which practicing; or any qualifications defined by the State in which practicing, unless licensure does not apply; or
  - (B) Must meet both of the following:
    - (1) Completed certification requirements to practice as an occupational therapy assistant established by a credentialing organization approved by the American Occupational Therapy Association.
    - (2) After January 1, 2010, meets the requirements in paragraph (b)(6)(i) of this section.
- (iii) After December 31, 1977 and on or before December 31, 2007—
  - (A) Completed certification requirements to practice as an occupational therapy assistant established by a credentialing organization approved by the American Occupational Therapy Association; or
  - (B) Completed the requirements to practice as an occupational therapy assistant applicable in the State in which practicing.
- (iv) On or before December 31, 1977—
  - (A) Had 2 years of appropriate experience as an occupational therapy assistant; and
  - (B) Had achieved a satisfactory grade on an occupational therapy assistant proficiency examination conducted, approved, or sponsored by the U.S. Public Health Service.
- (v) If educated outside the United States, on or after January 1, 2008—
  - (A) Graduated after successful completion of an occupational therapy assistant education program that is accredited as substantially equivalent to occupational therapist assistant entry level education in the United States by—
    - (1) The Accreditation Council for Occupational Therapy Education (ACOTE).
    - (2) Its successor organizations.
    - (3) The World Federation of Occupational Therapists.
    - (4) By a credentialing body approved by the American Occupational Therapy Association; and
    - (5) Successfully completed the entry level certification examination for occupational therapy assistants developed and administered by the National Board for Certification in Occupational Therapy, Inc. (NBCOT).

- (7) **Physical therapist.** A person who is licensed, if applicable, by the State in which practicing, unless licensure does not apply and meets one of the following requirements:
- (i) Graduated after successful completion of a physical therapist education program approved by one of the following:
    - (A) The Commission on Accreditation in Physical Therapy Education (CAPTE).
    - (B) Successor organizations of CAPTE.
    - (C) An education program outside the United States determined to be substantially equivalent to physical therapist entry level education in the United States by a credentials evaluation organization approved by the American Physical Therapy Association or an organization identified in 8 CFR 212.15(e) as it relates to physical therapists.
    - (D) Passed an examination for physical therapists approved by the State in which physical therapy services are provided.
  - (ii) On or before December 31, 2009—
    - (A) Graduated after successful completion of a physical therapy curriculum approved by the Commission on Accreditation in Physical Therapy Education (CAPTE); or
    - (B) Meets both of the following:
      - (1) Graduated after successful completion of an education program determined to be substantially equivalent to physical therapist entry level education in the United States by a credentials evaluation organization approved by the American Physical Therapy Association or identified in 8 CFR 212.15(e) as it relates to physical therapists.
      - (2) Passed an examination for physical therapists approved by the State in which physical therapy services are provided.
  - (iii) Before January 1, 2008—
    - (A) Graduated from a physical therapy curriculum approved by one of the following:
      - (1) The American Physical Therapy Association.
      - (2) The Committee on Allied Health Education and Accreditation of the American Medical Association.
      - (3) The Council on Medical Education of the American Medical Association and the American Physical Therapy Association.
  - (iv) On or before December 31, 1977 was licensed or qualified as a physical therapist and meets both of the following:
    - (A) Has 2 years of appropriate experience as a physical therapist.
    - (B) Has achieved a satisfactory grade on a proficiency examination conducted, approved, or sponsored by the U.S. Public Health Service.
  - (v) Before January 1, 1966—
    - (A) Was admitted to membership by the American Physical Therapy Association;

- (B) Was admitted to registration by the American Registry of Physical Therapists; and
  - (C) Graduated from a physical therapy curriculum in a 4-year college or university approved by a State department of education.
- (vi) Before January 1, 1966 was licensed or registered, and before January 1, 1970, had 15 years of fulltime experience in the treatment of illness or injury through the practice of physical therapy in which services were rendered under the order and direction of attending and referring doctors of medicine or osteopathy.
- (vii) If trained outside the United States before January 1, 2008, meets the following requirements:
  - (A) Was graduated since 1928 from a physical therapy curriculum approved in the country in which the curriculum was located and in which there is a member organization of the World Confederation for Physical Therapy.
  - (B) Meets the requirements for membership in a member organization of the World Confederation for Physical Therapy.
- (8) **Physical therapist assistant.** A person who is licensed, registered or certified as a physical therapist assistant, if applicable, by the State in which practicing, unless licensure does not apply and meets one of the following requirements:
  - (i) Graduated from a physical therapist assistant curriculum approved by the Commission on Accreditation in Physical Therapy Education of the American Physical Therapy Association; or if educated outside the United States or trained in the United States military, graduated from an education program determined to be substantially equivalent to physical therapist assistant entry level education in the United States by a credentials evaluation organization approved by the American Physical Therapy Association or identified at 8 CFR 212.15(e); and
  - (ii) Passed a national examination for physical therapist assistants.
    - (A) On or before December 31, 2009, meets one of the following:
      - (1) Is licensed, or otherwise regulated in the State in which practicing.
      - (2) In States where licensure or other regulations do not apply, graduated before December 31, 2009, from a 2-year college-level program approved by the American Physical Therapy Association and after January 1, 2010, meets the requirements of paragraph (b)(8) of this section.
      - (3) Before January 1, 2008, where licensure or other regulation does not apply, graduated from a 2-year college level program approved by the American Physical Therapy Association.
      - (4) On or before December 31, 1977, was licensed or qualified as a physical therapist assistant and has achieved a satisfactory grade on a proficiency examination conducted, approved, or sponsored by the U.S. Public Health Service.
- (9) Marriage and family therapist as defined at § 410.53.
- (10) Mental health counselor as defined at § 410.54.

- (c) **Personnel qualifications when no State licensing, certification or registration requirements exist.** If no State licensing laws, certification or registration requirements exist for the profession, the following requirements must be met:
  - (1) **Registered nurse.** A graduate of a school of professional nursing.
  - (2) **Licensed practical nurse.** A person who has completed a practical nursing program.
- (d) **Standard: Criminal background checks.**
  - (1) The hospice must obtain a criminal background check on all hospice employees who have direct patient contact or access to patient records. Hospice contracts must require that all contracted entities obtain criminal background checks on contracted employees who have direct patient contact or access to patient records.
  - (2) Criminal background checks must be obtained in accordance with State requirements. In the absence of State requirements, criminal background checks must be obtained within three months of the date of employment for all states that the individual has lived or worked in the past 3 years.

[73 FR 32204, June 5, 2008, as amended at 88 FR 79539, Nov. 16, 2023; 89 FR 64272, Aug. 6, 2024]

## **§ 418.116 Condition of participation: Compliance with Federal, State, and local laws and regulations related to the health and safety of patients.**

The hospice and its staff must operate and furnish services in compliance with all applicable Federal, State, and local laws and regulations related to the health and safety of patients. If State or local law provides for licensing of hospices, the hospice must be licensed.

- (a) **Standard: Multiple locations.** Every hospice must comply with the requirements of § 420.206 of this chapter regarding disclosure of ownership and control information. All hospice multiple locations must be approved by Medicare and licensed in accordance with State licensure laws, if applicable, before providing Medicare reimbursed services.
- (b) **Standard: Laboratory services.**
  - (1) If the hospice engages in laboratory testing other than assisting a patient in self-administering a test with an appliance that has been approved for that purpose by the FDA, the hospice must be in compliance with all applicable requirements of part 493 of this chapter.
  - (2) If the hospice chooses to refer specimens for laboratory testing to a reference laboratory, the reference laboratory must be certified in the appropriate specialties and subspecialties of services in accordance with the applicable requirements of part 493 of this chapter.

## **Subpart E [Reserved]**

## **Subpart F—Covered Services**

### **§ 418.200 Requirements for coverage.**

To be covered, hospice services must meet the following requirements. They must be reasonable and necessary for the palliation and management of the terminal illness as well as related conditions. The individual must elect hospice care in accordance with § 418.24. A plan of care must be established and periodically reviewed by the

attending physician, the medical director, and the interdisciplinary group of the hospice program as set forth in § 418.56. That plan of care must be established before hospice care is provided. The services provided must be consistent with the plan of care. A certification that the individual is terminally ill must be completed as set forth in section § 418.22.

[74 FR 39413, Aug. 6, 2009]

### § 418.202 Covered services.

All services must be performed by appropriately qualified personnel, but it is the nature of the service, rather than the qualification of the person who provides it, that determines the coverage category of the service. The following services are covered hospice services:

- (a) Nursing care provided by or under the supervision of a registered nurse.
- (b) Medical social services provided by a social worker under the direction of a physician.
- (c) Physicians' services performed by a physician as defined in § 410.20 of this chapter except that the services of the hospice medical director or the physician member of the interdisciplinary group must be performed by a doctor of medicine or osteopathy.
- (d) Counseling services provided to the terminally ill individual and the family members or other persons caring for the individual at home. Counseling, including dietary counseling, may be provided both for the purpose of training the individual's family or other caregiver to provide care, and for the purpose of helping the individual and those caring for him or her to adjust to the individual's approaching death.
- (e) Short-term inpatient care provided in a participating hospice inpatient unit, or a participating hospital or SNF, that additionally meets the standards in § 418.202 (a) and (e) regarding staffing and patient areas. Services provided in an inpatient setting must conform to the written plan of care. Inpatient care may be required for procedures necessary for pain control or acute or chronic symptom management.

Inpatient care may also be furnished as a means of providing respite for the individual's family or other persons caring for the individual at home. Respite care must be furnished as specified in § 418.108(b). Payment for inpatient care will be made at the rate appropriate to the level of care as specified in § 418.302.

- (f) **Medical appliances and supplies, including drugs and biologicals.** Only drugs as defined in section 1861(t) of the Act and which are used primarily for the relief of pain and symptom control related to the individual's terminal illness are covered. Appliances may include covered durable medical equipment as described in § 410.38 of this chapter as well as other self-help and personal comfort items related to the palliation or management of the patient's terminal illness. Equipment is provided by the hospice for use in the patient's home while he or she is under hospice care. Medical supplies include those that are part of the written plan of care and that are for palliation and management of the terminal or related conditions.
- (g) **Home health or hospice aide services furnished by qualified aides as designated in § 418.76 and homemaker services.** Home health aides (also known as hospice aides) may provide personal care services as defined in § 409.45(b) of this chapter. Aides may perform household services to maintain a safe and sanitary environment in areas of the home used by the patient, such as changing bed linens or light cleaning and laundering essential to the comfort and cleanliness of the patient. Aide services must be provided under the general supervision of a registered nurse. Homemaker services may include assistance in maintenance of a safe and healthy environment and services to enable the individual to carry out the treatment plan.



- (h) Physical therapy, occupational therapy and speech-language pathology services in addition to the services described in § 409.33 (b) and (c) of this chapter provided for purposes of symptom control or to enable the patient to maintain activities of daily living and basic functional skills.
- (i) Effective April 1, 1998, any other service that is specified in the patient's plan of care as reasonable and necessary for the palliation and management of the patient's terminal illness and related conditions and for which payment may otherwise be made under Medicare.

[48 FR 56026, Dec. 16, 1983, as amended at 51 FR 41351, Nov. 14, 1986; 55 FR 50835, Dec. 11, 1990; 59 FR 65498, Dec. 20, 1994; 70 FR 70547, Nov. 22, 2005; 73 FR 32220, June 5, 2008; 74 FR 39413, Aug. 6, 2009; 76 FR 47331, Aug. 4, 2011]

### § 418.204 Special coverage requirements.

- (a) **Periods of crisis.** Nursing care may be covered on a continuous basis for as much as 24 hours a day during periods of crisis as necessary to maintain an individual at home. Either homemaker or home health aide (also known as hospice aide) services or both may be covered on a 24-hour continuous basis during periods of crisis but care during these periods must be predominantly nursing care. A period of crisis is a period in which the individual requires continuous care to achieve palliation and management of acute medical symptoms.
- (b) **Respite care.**
  - (1) Respite care is short-term inpatient care provided to the individual only when necessary to relieve the family members or other persons caring for the individual.
  - (2) Respite care may be provided only on an occasional basis and may not be reimbursed for more than five consecutive days at a time.
- (c) **Bereavement counseling.** Bereavement counseling is a required hospice service but it is not reimbursable.

[48 FR 56026, Dec. 16, 1983, as amended at 55 FR 50835, Dec. 11, 1990; 74 FR 39413, Aug. 6, 2009; 85 FR 19289, Apr. 6, 2020; 88 FR 51199, Aug. 2, 2023]

### § 418.205 Special requirements for hospice pre-election evaluation and counseling services.

- (a) **Definition.** As used in this section the following definition applies.

**Terminal illness** has the same meaning as defined in § 418.3.

- (b) **General.** Effective January 1, 2005, payment for hospice pre-election evaluation and counseling services as specified in § 418.304(d) may be made to a hospice on behalf of a Medicare beneficiary if the requirements of this section are met.
  - (1) **The beneficiary.** The beneficiary:
    - (i) Has been diagnosed as having a terminal illness as defined in § 418.3.
    - (ii) Has not made a hospice election.
    - (iii) Has not previously received hospice pre-election evaluation and consultation services specified under this section.

- (2) **Services provided.** The hospice pre-election services include an evaluation of an individual's need for pain and symptom management and counseling regarding hospice and other care options. In addition, the services may include advising the individual regarding advanced care planning.
- (3) **Provision of pre-election hospice services.**
  - (i) The services must be furnished by a physician.
  - (ii) The physician furnishing these services must be an employee or medical director of the hospice billing for this service.
  - (iii) The services cannot be furnished by hospice personnel other than employed physicians, such as but not limited to nurse practitioners, nurses, or social workers, physicians under contractual arrangements with the hospice or by the beneficiary's physician, if that physician is not an employee of the hospice.
  - (iv) If the beneficiary's attending physician is also the medical director or a physician employee of the hospice, the attending physician may not provide nor may the hospice bill for this service because that physician already possesses the expertise necessary to furnish end-of-life evaluation and management, and counseling services.
- (4) **Documentation.**
  - (i) If the individual's physician initiates the request for services of the hospice medical director or physician, appropriate documentation is required.
  - (ii) The request or referral must be in writing, and the hospice medical director or physician employee is expected to provide a written note on the patient's medical record.
  - (iii) The hospice agency employing the physician providing these services is required to maintain a written record of the services furnished.
  - (iv) If the services are initiated by the beneficiary, the hospice agency is required to maintain a record of the services and documentation that communication between the hospice medical director or physician and the beneficiary's physician occurs, with the beneficiary's permission, to the extent necessary to ensure continuity of care.

[69 FR 66425, Nov. 15, 2004]

## Subpart G—Payment for Hospice Care

### § 418.301 Basic rules.

- (a) Medicare payment for covered hospice care is made in accordance with the method set forth in § 418.302.
- (b) Medicare reimbursement to a hospice in a cap period is limited to a cap amount specified in § 418.309.
- (c) The hospice may not charge a patient for services for which the patient is entitled to have payment made under Medicare or for services for which the patient would be entitled to payment, as described in § 489.21 of this chapter.

[48 FR 56026, Dec. 16, 1983, as amended at 56 FR 26919, June 12, 1991; 70 FR 70547, Nov. 22, 2005]

## § 418.302 Payment procedures for hospice care.

- (a) CMS establishes payment amounts for specific categories of covered hospice care.
- (b) Payment amounts are determined within each of the following categories:
  - (1) **Routine home care day.** A routine home care day is a day on which an individual who has elected to receive hospice care is at home and is not receiving continuous care as defined in paragraph (b)(2) of this section.
    - (i) **Service intensity add-on.** Routine home care days that occur during the last 7 days of a hospice election ending with a patient discharged due to death are eligible for a service intensity add-on payment.
    - (ii) The service intensity add-on payment shall be equal to the continuous home care hourly payment rate, as described in paragraph (e)(4) of this section, multiplied by the amount of direct patient care actually provided by a RN and/or social worker, up to 4 hours total per day.
  - (2) **Continuous home care day.** A continuous home care day is a day on which an individual who has elected to receive hospice care is not in an inpatient facility and receives hospice care consisting predominantly of nursing care on a continuous basis at home. Home health aide (also known as a hospice aide) or homemaker services or both may also be provided on a continuous basis. Continuous home care is only furnished during brief periods of crisis as described in § 418.204(a) and only as necessary to maintain the terminally ill patient at home.
  - (3) **Inpatient respite care day.** An inpatient respite care day is a day on which the individual who has elected hospice care receives care in an approved facility on a short-term basis for respite.
  - (4) **General inpatient care day.** A general inpatient care day is a day on which an individual who has elected hospice care receives general inpatient care in an inpatient facility for pain control or acute or chronic symptom management which cannot be managed in other settings.
- (c) The payment amounts for the categories of hospice care are fixed payment rates that are established by CMS in accordance with the procedures described in § 418.306. Payment rates are determined for the following categories:
  - (1) Routine home care.
  - (2) Continuous home care.
  - (3) Inpatient respite care.
  - (4) General inpatient care.
- (d)
  - (1) The Medicare Administrative Contractor reimburses the hospice its appropriate payment amount for each day for which an eligible Medicare beneficiary is under the hospice's care.
  - (2) Effective December 8, 2003, if a hospice makes arrangements with another hospice to provide services under the circumstances specified in section 1861(dd)(5)(D) of the Act, the Medicare Administrative Contractor reimburses the hospice for which the beneficiary has made an election as described in paragraph (d)(1) of this section.
- (e) The Medicare Administrative Contractor makes payment according to the following procedures:

- (1) Payment is made to the hospice for each day during which the beneficiary is eligible and under the care of the hospice, regardless of the amount of services furnished on any given day (except as set out in paragraph (b)(1)(i) of this section).
  - (2) Payment is made for only one of the categories of hospice care described in § 418.302(b) for any particular day.
  - (3) On any day on which the beneficiary is not an inpatient, the hospice is paid the routine home care rate, unless the patient receives continuous care as defined in paragraph (b)(2) of this section for a period of at least 8 hours. In that case, a portion of the continuous care day rate is paid in accordance with paragraph (e)(4) of this section.
  - (4) The hospice payment on a continuous care day varies depending on the number of hours of continuous services provided. The continuous home care rate is divided by 24 to yield an hourly rate. The number of hours of continuous care provided during a continuous home care day is then multiplied by the hourly rate to yield the continuous home care payment for that day. A minimum of 8 hours of care must be furnished on a particular day to qualify for the continuous home care rate.
  - (5) Subject to the limitations described in paragraph (f) of this section, on any day on which the beneficiary is an inpatient in an approved facility for inpatient care, the appropriate inpatient rate (general or respite) is paid depending on the category of care furnished. The inpatient rate (general or respite) is paid for the date of admission and all subsequent inpatient days, except the day on which the patient is discharged. For the day of discharge, the appropriate home care rate is paid unless the patient dies as an inpatient. In the case where the beneficiary is discharged deceased, the inpatient rate (general or respite) is paid for the discharge day. Payment for inpatient respite care is subject to the requirement that it may not be provided consecutively for more than 5 days at a time. Payment for the sixth and any subsequent day of respite care is made at the routine home care rate.
- (f) Payment for inpatient care is limited as follows:
- (1) The total payment to the hospice for inpatient care (general or respite) is subject to a limitation that total inpatient care days for Medicare patients not exceed 20 percent of the total days for which these patients had elected hospice care.
  - (2) At the end of a cap period, the Medicare Administrative Contractor calculates a limitation on payment for inpatient care to ensure that Medicare payment is not made for days of inpatient care in excess of 20 percent of the total number of days of hospice care furnished to Medicare patients. Only inpatient days that were provided and billed as general inpatient or respite days are counted as inpatient days when computing the inpatient cap.
  - (3) If the number of days of inpatient care furnished to Medicare patients is equal to or less than 20 percent of the total days of hospice care to Medicare patients, no adjustment is necessary. Overall payments to a hospice are subject to the cap amount specified in § 418.309.
  - (4) If the number of days of inpatient care furnished to Medicare patients exceeds 20 percent of the total days of hospice care to Medicare patients, the total payment for inpatient care is determined in accordance with the procedures specified in paragraph (f)(5) of this section. That amount is compared to actual payments for inpatient care, and any excess reimbursement must be refunded by the hospice. Overall payments to the hospice are subject to the cap amount specified in § 418.309.

- (5) If a hospice exceeds the number of inpatient care days described in paragraph (f)(4), the total payment for inpatient care is determined as follows:
  - (i) Calculate the ratio of the maximum number of allowable inpatient days to the actual number of inpatient care days furnished by the hospice to Medicare patients.
  - (ii) Multiply this ratio by the total reimbursement for inpatient care made by the Medicare Administrative Contractor.
  - (iii) Multiply the number of actual inpatient days in excess of the limitation by the routine home care rate.
  - (iv) Add the amounts calculated in paragraphs (f)(5)(ii) and (iii) of this section.
- (g) Payment for routine home care, continuous home care, general inpatient care and inpatient respite care is made on the basis of the geographic location where the services are provided.

[48 FR 56026, Dec. 16, 1983, as amended at 56 FR 26919, June 12, 1991; 70 FR 45145, Aug. 4, 2005; 70 FR 70547, Nov. 22, 2005; 72 FR 50228, Aug. 31, 2007; 74 FR 39414, Aug. 6, 2009; 80 FR 47206, Aug. 6, 2015]

#### **§ 418.304 Payment for physician, and nurse practitioner, and physician assistant services.**

- (a) The following services performed by hospice physicians and nurse practitioners are included in the rates described in § 418.302:
  - (1) General supervisory services of the medical director.
  - (2) Participation in the establishment of plans of care, supervision of care and services, periodic review and updating of plans of care, and establishment of governing policies by the physician member of the interdisciplinary group.
- (b) For services not described in paragraph (a) of this section, a specified Medicare contractor pays the hospice an amount equivalent to 100 percent of the physician fee schedule for those physician services furnished by hospice employees or under arrangements with the hospice. Reimbursement for these physician services is included in the amount subject to the hospice payment limit described in § 418.309. Services furnished voluntarily by physicians are not reimbursable.
- (c) Services of the patient's attending physician, if he or she is not an employee of the hospice or providing services under arrangements with the hospice, are not considered hospice services and are not included in the amount subject to the hospice payment limit described in § 418.309. These services are paid by the carrier under the procedures in subpart B, part 414 of this chapter.
- (d) **Payment for hospice pre-election evaluation and counseling services.** The intermediary makes payment to the hospice for the services established in § 418.205. Payment for this service is set at an amount established under the physician fee schedule, for an office or other outpatient visit for evaluation and management associated with presenting problems of moderate severity and requiring medical decision-making of low complexity other than the portion of the amount attributable to the practice expense component. Payment for this pre-election service does not count towards the hospice cap amount.
- (e)

- (1) Effective December 8, 2003, Medicare pays for attending physician services provided by nurse practitioners to Medicare beneficiaries who have elected the hospice benefit and who have selected a nurse practitioner as their attending physician. This applies to nurse practitioners without regard to whether they are hospice employees.
- (2) Nurse practitioners may bill and receive payment for services only if the—
  - (i) Nurse practitioner is the beneficiary's attending physician as defined in § 418.3;
  - (ii) Services are medically reasonable and necessary;
  - (iii) Services are performed by a physician in the absence of the nurse practitioner; and
  - (iv) Services are not related to the certification of terminal illness specified in § 418.22.
- (3) Payment for nurse practitioner services are made at 85 percent of the physician fee schedule amount.

(f)

- (1) Effective January 1, 2019, Medicare pays for attending physician services provided by physician assistants to Medicare beneficiaries who have elected the hospice benefit and who have selected a physician assistant as their attending physician. This applies to physician assistants without regard to whether they are hospice employees.
- (2) The employer or a contractor of a physician assistant must bill and receive payment for physician assistant services only if the—
  - (i) Physician assistant is the beneficiary's attending physician as defined in § 418.3;
  - (ii) Services are medically reasonable and necessary;
  - (iii) Services are performed by a physician in the absence of the physician assistant and, the physician assistant services are furnished under the general supervision of a physician; and
  - (iv) Services are not related to the certification of terminal illness specified in § 418.22.
- (3) The payment amount for physician assistant services when serving as the attending physician for hospice patients is 85 percent of what a physician is paid under the Medicare physician fee schedule.

[48 FR 56026, Dec. 16, 1983, as amended at 69 FR 66426, Nov. 15, 2004; 70 FR 45145, Aug. 4, 2005; 70 FR 70547, Nov. 22, 2005; 83 FR 38655, Aug. 6, 2018]

### § 418.306 Annual update of the payment rates and adjustment for area wage differences.

- (a) **Applicability.** CMS establishes payment rates for each of the categories of hospice care described in § 418.302(b). The rates are established using the methodology described in section 1814(i)(1)(C) of the Act and in accordance with section 1814(i)(6)(D) of the Act.
- (b) **Annual update of the payment rates.** The payment rates for routine home care and other services included in hospice care are the payment rates in effect under this paragraph during the previous fiscal year increased by the hospice payment update percentage increase (as defined in sections 1814(i)(1)(C) of the Act), applicable to discharges occurring in the fiscal year.

- (1) For fiscal year 2014 and subsequent fiscal years, in accordance with section 1814(i)(5)(A)(i) of the Act, in the case of a Medicare-certified hospice that submits hospice quality data, as specified by the Secretary, the payment rates are equal to the rates for the previous fiscal year increased by the applicable hospice payment update percentage increase.
- (2) For fiscal years 2014 and through 2023, in accordance with section 1814(i)(5)(A)(i) of the Act, in the case of a Medicare-certified hospice that does not submit hospice quality data, as specified by the Secretary, the payment rates are equal to the rates for the previous fiscal year increased by the applicable hospice payment update percentage increase, minus 2 percentage points. Beginning with fiscal year 2024 and subsequent fiscal years, the reduction increases to 4 percentage points. Any reduction of the percentage change will apply only to the fiscal year involved and will not be taken into account in computing the payment amounts for a subsequent fiscal year.

(c) **Adjustment for wage differences.**

- (1) Each hospice's labor market is determined based on definitions of Metropolitan Statistical Areas (MSAs) issued by OMB. CMS will issue annually, in the FEDERAL REGISTER, a hospice wage index based on the most current available CMS hospital wage data, including changes to the definition of MSAs. The urban and rural area geographic classifications are defined in § 412.64(b)(1)(ii)(A) through (C) of this chapter. The payment rates established by CMS are adjusted by the Medicare contractor to reflect local differences in wages according to the revised wage data.
- (2) Beginning on October 1, 2022, CMS applies a cap on decreases to the hospice wage index such that the wage index applied to a geographic area is not less than 95 percent of the wage index applied to that geographic area in the prior fiscal year.

(d) **Federal Register notices.** CMS publishes as a notice in the FEDERAL REGISTER any proposal to change the methodology for determining the payment rates.

[56 FR 26919, June 12, 1991, as amended at 59 FR 26960, May 25, 1994; 62 FR 42882, Aug. 8, 1997; 70 FR 70548, Nov. 22, 2005; 73 FR 46486, Aug. 8, 2008; 79 FR 50509, Aug. 22, 2014; 80 FR 47207, Aug. 6, 2015; 86 FR 42605, Aug. 4, 2021; 87 FR 45702, July 29, 2022]

### § 418.307 Periodic interim payments.

Subject to the provisions of § 413.64(h) of this chapter, a hospice may elect to receive periodic interim payments (PIP) effective with claims received on or after July 1, 1987. Payment is made biweekly under the PIP method unless the hospice requests a longer fixed interval (not to exceed one month) between payments. The biweekly interim payment amount is based on the total estimated Medicare payments for the reporting period (as described in §§ 418.302-418.306). Each payment is made 2 weeks after the end of a biweekly period of service as described in § 413.64(h)(5) of this chapter. Under certain circumstances that are described in § 413.64(g) of this chapter, a hospice that is not receiving PIP may request an accelerated payment.

[59 FR 36713, July 19, 1994]

### § 418.308 Limitation on the amount of hospice payments.

- (a) Except as specified in paragraph (b) of this section, the total Medicare payment to a hospice for care furnished during a cap period is limited by the hospice cap amount specified in § 418.309.

- (b) Until October 1, 1986, payment to a hospice that began operation before January 1, 1975 is not limited by the amount of the hospice cap specified in § 418.309.
- (c) The hospice must file its aggregate cap determination notice with its Medicare contractor no later than 5 months after the end of the cap year and remit any overpayment due at that time. Hospices shall file the aggregate cap using data no earlier than 3 months after the end of the cap period. The Medicare contractor will notify the hospice of the final determination of program reimbursement in accordance with procedures similar to those described in § 405.1803 of this chapter. If a provider fails to file its self-determined cap determination with its Medicare contractor within 5 months after the cap year, payments to the hospice will be suspended in whole or in part, until a self-determined cap determination is filed with the Medicare contractor, in accordance with § 405.371(e) of this chapter.
- (d) Payments made to a hospice during a cap period that exceed the cap amount are overpayments and must be refunded.

[48 FR 56026, Dec. 16, 1983; 48 FR 57282, Dec. 29, 1983, as amended at 79 FR 50509, Aug. 22, 2014; 80 FR 47207, Aug. 6, 2015]

### § 418.309 Hospice aggregate cap.

A hospice's aggregate cap is calculated by multiplying the adjusted cap amount (determined in paragraph (a) of this section) by the number of Medicare beneficiaries, as determined by one of two methodologies for determining the number of Medicare beneficiaries for a given cap year described in paragraphs (b) and (c) of this section.

- (a) **Cap Amount.** The cap amount was set at \$6,500 in 1983 and is updated using one of two methodologies described in paragraphs (a)(1) and (a)(2) of this section.
  - (1) For accounting years that end on or before September 30, 2016 and end on or after October 1, 2033, the cap amount is adjusted for inflation by using the percentage change in the medical care expenditure category of the Consumer Price Index (CPI) for urban consumers that is published by the Bureau of Labor Statistics. This adjustment is made using the change in the CPI from March 1984 to the fifth month of the cap year.
  - (2) For accounting years that end after September 30, 2016, and before October 1, 2033, the cap amount is the cap amount for the preceding accounting year updated by the percentage update to payment rates for hospice care for services furnished during the fiscal year beginning on the October 1 preceding the beginning of the accounting year as determined pursuant to section 1814(i)(1)(C) of the Act (including the application of any productivity or other adjustments to the hospice percentage update).
- (b) **Streamlined methodology defined.** A hospice's aggregate cap is calculated by multiplying the adjusted cap amount determined in paragraph (a) of this section by the number of Medicare beneficiaries as determined in paragraphs (b)(1) and (2) of this section. For purposes of the streamlined methodology calculation—
  - (1) In the case in which a beneficiary received care from only one hospice, the hospice includes in its number of Medicare beneficiaries those Medicare beneficiaries who have not previously been included in the calculation of any hospice cap, and who have filed an election to receive hospice care in accordance with § 418.24 during the cap period as defined in § 418.3, using the best data available at the time of the calculation.



(2) In the case in which a beneficiary received care from more than one hospice, each hospice includes in its number of Medicare beneficiaries only that fraction which represents the portion of a patient's total days of care in all hospices and all years that was spent in that hospice in that cap year, using the best data available at the time of the calculation. The aggregate cap calculation for a given cap year may be adjusted after the calculation for that year based on updated data.

(c) ***Patient-by-patient proportional methodology defined.*** A hospice's aggregate cap is calculated by multiplying the adjusted cap amount determined in paragraph (a) of this section by the number of Medicare beneficiaries as described in paragraphs (c)(1) and (2) of this section. For the purposes of the patient-by-patient proportional methodology—

(1) A hospice includes in its number of Medicare beneficiaries only that fraction which represents the portion of a patient's total days of care in all hospices and all years that was spent in that hospice in that cap year, using the best data available at the time of the calculation. The total number of Medicare beneficiaries for a given hospice's cap year is determined by summing the whole or fractional share of each Medicare beneficiary that received hospice care during the cap year, from that hospice.

(2) The aggregate cap calculation for a given cap year may be adjusted after the calculation for that year based on updated data.

(d) ***Application of methodologies.***

(1) For cap years ending October 31, 2011 and for prior cap years, a hospice's aggregate cap is calculated using the streamlined methodology described in paragraph (b) of this section, subject to the following:

(i) A hospice that has not received a cap determination for a cap year ending on or before October 31, 2011 as of October 1, 2011, may elect to have its final cap determination for such cap years calculated using the patient-by-patient proportional methodology described in paragraph (c) of this section; or

(ii) A hospice that has filed a timely appeal regarding the methodology used for determining the number of Medicare beneficiaries in its cap calculation for any cap year is deemed to have elected that its cap determination for the challenged year, and all subsequent cap years, be calculated using the patient-by-patient proportional methodology described in paragraph (c) of this section.

(2) For cap years ending October 31, 2012, and all subsequent cap years, a hospice's aggregate cap is calculated using the patient-by-patient proportional methodology described in paragraph (c) of this section, subject to the following:

(i) A hospice that has had its cap calculated using the patient-by-patient proportional methodology for any cap year(s) prior to the 2012 cap year is not eligible to elect the streamlined methodology, and must continue to have the patient-by-patient proportional methodology used to determine the number of Medicare beneficiaries in a given cap year.

(ii) A hospice that is eligible to make a one-time election to have its cap calculated using the streamlined methodology must make that election no later than 60 days after receipt of its 2012 cap determination. A hospice's election to have its cap calculated using the streamlined methodology would remain in effect unless:

- (A) The hospice subsequently submits a written election to change the methodology used in its cap determination to the patient-by-patient proportional methodology; or
  - (B) The hospice appeals the streamlined methodology used to determine the number of Medicare beneficiaries used in the aggregate cap calculation.
- (3) If a hospice that elected to have its aggregate cap calculated using the streamlined methodology under paragraph (d)(2)(ii) of this section subsequently elects the patient-by-patient proportional methodology or appeals the streamlined methodology, under paragraph (d)(2)(ii)(A) or (B) of this section, the hospice's aggregate cap determination for that cap year and all subsequent cap years is to be calculated using the patient-by-patient proportional methodology. As such, past cap year determinations may be adjusted to prevent the over-counting of beneficiaries, subject to existing reopening regulations.

[48 FR 56026, Dec. 16, 1983, as amended at 76 FR 47332, Aug. 4, 2011; 80 FR 47207, Aug. 6, 2015; 83 FR 38655, Aug. 6, 2018; 86 FR 42606, Aug. 4, 2021; 88 FR 51199, Aug. 2, 2023; 89 FR 64272, Aug. 6, 2024]

### **§ 418.310 Reporting and recordkeeping requirements.**

Hospices must provide reports and keep records as the Secretary determines necessary to administer the program.

### **§ 418.311 Administrative appeals.**

A hospice that believes its payments have not been properly determined in accordance with these regulations may request a review from the intermediary or the Provider Reimbursement Review Board (PRRB) if the amount in controversy is at least \$1,000 or \$10,000, respectively. In such a case, the procedure in 42 CFR part 405, subpart R, will be followed to the extent that it is applicable. The PRRB, subject to review by the Secretary under § 405.1875 of this chapter, shall have the authority to determine the issues raised. The methods and standards for the calculation of the statutorily defined payment rates by CMS are not subject to appeal.

[74 FR 39414, Aug. 6, 2009, as amended at 78 FR 48281, Aug. 7, 2013]

### **§ 418.312 Data submission requirements under the hospice quality reporting program.**

- (a) **General rule.** Except as provided in paragraph (g) of this section, Medicare-certified hospices must submit to CMS data on measures selected under section 1814(i)(5)(C) of the Act in a form and manner, and at a time, specified by the Secretary.
- (b) **Submission of Hospice Quality Reporting Program data.**
  - (1) Hospices are required to complete and submit a standardized set of items for each patient to capture patient-level data, regardless of payer or patient age. The standardized set of items must be completed no less frequently than at admission, the hospice update visit (HUV), and discharge, as directed in the associated guidance manual and required by the Hospice Quality Reporting Program. Definitions for changes in patient condition that warrant updated assessment, as well as the data elements to be completed for each applicable change in patient condition, are to be provided in sub-regulatory guidance for the current standardized hospice instrument.
  - (2) Administrative data, such as Medicare claims data, used for hospice quality measures to capture services throughout the hospice stay, are required and fulfill the HQRP requirements for § 418.306(b).

- (3) CMS may remove a quality measure from the Hospice QRP based on one or more of the following factors:
  - (i) Measure performance among hospices is so high and unvarying that meaningful distinctions in improvements in performance can no longer be made.
  - (ii) Performance or improvement on a measure does not result in better patient outcomes.
  - (iii) A measure does not align with current clinical guidelines or practice.
  - (iv) The availability of a more broadly applicable (across settings, populations, or conditions) measure for the particular topic.
  - (v) The availability of a measure that is more proximal in time to desired patient outcomes for the particular topic.
  - (vi) The availability of a measure that is more strongly associated with desired patient outcomes for the particular topic.
  - (vii) Collection or public reporting of a measure leads to negative unintended consequences other than patient harm.
  - (viii) The costs associated with a measure outweigh the benefit of its continued use in the program.
- (c) A hospice that receives notice of its CMS certification number before November 1 of the calendar year before the fiscal year for which a payment determination will be made must submit data for the calendar year.
- (d) Medicare-certified hospices must contract with CMS-approved vendors to collect the CAHPS® Hospice Survey data on their behalf and submit the data to the Hospice CAHPS® Data Center.
- (e) If the hospice's total, annual, unique, survey-eligible, deceased patient count for the prior calendar year is less than 50 patients, the hospice is eligible to be exempt from the CAHPS® Hospice Survey reporting requirements in the current calendar year. In order to qualify for this exemption the hospice must submit to CMS its total, annual, unique, survey-eligible, deceased patient count for the prior calendar year.
- (f) Vendors that want to become CMS-approved CAHPS® Hospice Survey vendors must meet the minimum business requirements. Survey vendors must have been in business for a minimum of 4 years, have conducted surveys in the approved survey mode for a minimum of 3 years, and have conducted surveys of individual patients for a minimum of 2 years. For Hospice CAHPS®, a "survey of individual patients" is defined as the collection of data from at least 600 individual patients selected by statistical sampling methods, and the data collected are used for statistical purposes. Vendors may not use home-based or virtual interviewers to conduct the CAHPS® Hospice Survey, nor may they conduct any survey administration processes (for example, mailings) from a residence.
- (g) No organization, firm, or business that owns, operates, or provides staffing for a hospice is permitted to administer its own Hospice CAHPS® survey or administer the survey on behalf of any other hospice in the capacity as a Hospice CAHPS® survey vendor. Such organizations will not be approved by CMS as CAHPS® Hospice Survey vendors.
- (h) *Reconsiderations and appeals of Hospice Quality Reporting Program decisions.*

- (1) A hospice may request reconsideration of a decision by CMS that the hospice has not met the requirements of the Hospice Quality Reporting Program for a particular reporting period. A hospice must submit a reconsideration request to CMS no later than 30 days from the date identified on the annual payment update notification provided to the hospice.
- (2) Reconsideration request submission requirements are available on the CMS Hospice Quality Reporting Web site on CMS.gov.
- (3) A hospice that is dissatisfied with a decision made by CMS on its reconsideration request may file an appeal with the Provider Reimbursement Review Board under part 405, subpart R of this chapter.

(i) **Exemptions and extensions requirements.**

- (1) A hospice may request and CMS may grant exemptions or extensions to the reporting requirements under paragraph (b) of this section for one or more quarters, when there are certain extraordinary circumstances beyond the control of the hospice.
- (2) A hospice requesting an exemption or extension must do so within 90 days of the date that the extraordinary circumstances occurred by sending an email to CMS Hospice QRP Reconsiderations at [HospiceQRPreconsiderations@cms.hhs.gov](mailto:HospiceQRPreconsiderations@cms.hhs.gov) that contains all of the following information:
  - (i) Hospice CMS Certification Number (CCN).
  - (ii) Hospice Business Name.
  - (iii) Hospice Business Address.
  - (iv) CEO or CEO-designated personnel contact information including name, title, telephone number, email address, and mailing address (the address must be a physical address, not a post office box).
  - (v) Hospice's reason for requesting the exemption or extension.
  - (vi) Evidence of the impact of extraordinary circumstances beyond the hospice's control, including, but not limited to photographs, newspaper, other media articles, or independent sources attesting to the incident that can be reasonably corroborated. Include dates of occurrence and other documentation that may support the rationale for seeking extension or exemption.
  - (vii) Date when the hospice believes it will be able to again submit data under paragraph (b) of this section and a justification for the proposed date.
- (3) CMS may grant exemptions or extensions to hospices without a request if it determines that one or more of the following has occurred:
  - (i) An extraordinary circumstance, such as an act of nature including a pandemic, affects an entire region or locale.
  - (ii) A systemic problem with one of CMS' data collection systems directly affect the ability of a hospice to submit data under paragraph (b) of this section.

(j) **Data completion thresholds.**

- (1) Hospices must meet or exceed data submission threshold set at 90 percent of all required HIS or successor instrument records within 30-days of the beneficiary's admission or discharge and submitted through the CMS designated data submission systems.

- (2) A hospice must meet or exceed the data submission compliance threshold in paragraph (j)(1) of this section to avoid receiving a 4-percentage point reduction to its annual payment update for a given FY as described under § 418.306(b)(2) of this chapter.

[79 FR 50510, Aug. 22, 2014, as amended at 85 FR 53680, Aug. 31, 2020; 86 FR 42606, Aug. 4, 2021; 88 FR 51199, Aug. 2, 2023; 89 FR 64272, Aug. 6, 2024; 90 FR 37431, Aug. 5, 2025]

## Subpart H—Coinsurance

### § 418.400 Individual liability for coinsurance for hospice care.

An individual who has filed an election for hospice care in accordance with § 418.24 is liable for the following coinsurance payments. Hospices may charge individuals the applicable coinsurance amounts.

- (a) **Drugs and biologicals.** An individual is liable for a coinsurance payment for each palliative drug and biological prescription furnished by the hospice while the individual is not an inpatient. The amount of coinsurance for each prescription approximates 5 percent of the cost of the drug or biological to the hospice determined in accordance with the drug copayment schedule established by the hospice, except that the amount of coinsurance for each prescription may not exceed \$5. The cost of the drug or biological may not exceed what a prudent buyer would pay in similar circumstances. The drug copayment schedule must be reviewed for reasonableness and approved by the intermediary before it is used.
- (b) **Respite care.**
- (1) The amount of coinsurance for each respite care day is equal to 5 percent of the payment made by CMS for a respite care day.
- (2) The amount of the individual's coinsurance liability for respite care during a hospice coinsurance period may not exceed the inpatient hospital deductible applicable for the year in which the hospice coinsurance period began.
- (3) The individual hospice coinsurance period—
- (i) Begins on the first day an election filed in accordance with § 418.24 is in effect for the beneficiary; and
- (ii) Ends with the close of the first period of 14 consecutive days on each of which an election is not in effect for the beneficiary.

### § 418.402 Individual liability for services that are not considered hospice care.

Medicare payment to the hospice discharges an individual's liability for payment for all services, other than the hospice coinsurance amounts described in § 418.400, that are considered covered hospice care (as described in § 418.202). The individual is liable for the Medicare deductibles and coinsurance payments and for the difference between the reasonable and actual charge on unassigned claims on other covered services that are not considered hospice care. Examples of services not considered hospice care include: Services furnished before or after a hospice election period; services of the individual's attending physician, if the attending physician is not an employee of or working under an arrangement with the hospice; or Medicare services received for the treatment of an illness or injury not related to the individual's terminal condition.

**§ 418.405 Effect of coinsurance liability on Medicare payment.**

The Medicare payment rates established by CMS in accordance with § 418.306 are not reduced when the individual is liable for coinsurance payments. Instead, when establishing the payment rates, CMS offsets the estimated cost of services by an estimate of average coinsurance amounts hospices collect.

[56 FR 26919, June 12, 1991]  
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